

ENGLISH FOR ACCURATE PHARMACY COMMUNICATION

Pharmacy Technician English

Check the detail. Explain the next step.



Phil Stilwell

© 2026 Phil Stilwell. All rights reserved.

No part of this publication may be reproduced, stored in a retrieval system, or transmitted in any form or by any means without prior written permission from the author, except as permitted by law.

Eight lessons. Eleven extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
220 extended-dialogue turns / 11 transfer exchanges
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For pharmacy technicians and pharmacy support staff communicating with patients, pharmacists, prescribers, insurers, and suppliers.

2026 edition

Eight scenarios. Eight skills.

Clarify orders, claims, renewals, stock, label questions, private collection, recalls, and readiness messages. Three further cases address days-supply calculations, a temperature excursion, and a mid-cycle medicine-pack change.

01 Prescription intake without assumptions 4

A familiar label does not resolve an unclear strength | Identify an ambiguous order field and arrange pharmacist clarification without selecting a strength from memory or appearance.

02 Insurance claims in plain English 13

Prior authorization required is not a final exclusion | Explain an insurance response accurately and connect the patient with an accepted status query without promising coverage.

03 Refill requests and response windows 22

A status call is not a promise of supply | Explain an unresolved renewal and make a reliable communication commitment without promising prescriber response or medicine availability.

04 Stock enquiries and delivery estimates 31

Wednesday is an estimate, not a collection promise | Explain a stock shortage, qualify a supplier forecast, and route alternative-treatment questions without choosing a substitute.

05 Pickup questions and pharmacist consultations 40

The label question has a named pharmacist | Recognize an auxiliary-label question and complete a clear pharmacist handoff without interpreting or changing the instruction.

06 Private collection and contact preferences 49

A neighbor at the counter | Explain a private verification step without accusing the collector or disclosing a patient medication list.

07 Precise inventory and quality queries 58

AB41 is not AB14 | Read back product identifiers and distinguish an unverified stock match from a confirmed recall assessment.

08 Corrected messages and shift handoffs 67

Correcting a ready message | Correct an inaccurate status promptly, acknowledge its effect, and hand over a named follow-up without promising collection.

Communication field notes	3
Three more workplace conversations	76
Answers and explanations	85
Fast-access phrase pages	104
Vocabulary index	106
Sources and publication notes	114

Small wording differences carry important meaning.

Pharmacy communication connects exact product information with patient expectations. Keep a request separate from approval, a delivery estimate separate from stock on hand, and a queue entry separate from completed pharmacist review. Give the person a useful next step without inventing a clinical answer.

Do not fill a gap with a guess

A familiar package, remembered color, or usual prescription does not resolve an unclear current order. Identify the missing detail and use the appropriate pharmacist clarification route.

"The strength field is unclear; the pharmacist needs to clarify it before we can confirm the order."

Translate status, not certainty

Explain what a claim response or work-queue status actually means. A request for prior authorization does not by itself establish permanent exclusion, approval, or a confirmed patient price.

"The plan is asking for prior authorization; no decision has arrived yet."

Keep a handoff accountable

Name the person who accepted the question, what remains unresolved, and the next contact you can genuinely commit to. Do not turn an update window into a promise of supply.

"Jo will call between three and four with the status, not a guaranteed collection time."

Correct a wrong message plainly

If an earlier readiness statement was wrong, say what was wrong, give the verified status, and act promptly on its practical effect. Preserve the correction and actual contact outcome in the record.

"I told you it was ready; that was incorrect. Pharmacist review is still pending."

PROFESSIONAL BOUNDARIES

All cases, calculations, and product limits are fictional language exercises, not patient-specific directions or clinical certification. Follow local law, authorized duties, pharmacist direction, privacy procedures, and actual recall notices. Urgent concerns need the appropriate prompt response, not a routine update window. US and UK sources provide context, not interchangeable rules. This book does not teach prescribing, dose selection, or clinical counseling.

A familiar label does not resolve an unclear strength

A Read the situation

Identify an ambiguous order field and arrange pharmacist clarification without selecting a strength from memory or appearance.

Patient Ellis asks technician Nia to complete a prescription order whose strength field is unclear. Ellis remembers a blue label and says the prescriber usually orders the same thing. Nia cannot select the strength. Pharmacist Maya can take the clarification, but no completion time is known. Nia must distinguish remembered information from verified current instructions, explain the unresolved field, and route the question. No medicine name, dose, or numerical strength is provided in this teaching case, and the exercise does not ask the learner to infer one.

PEOPLE / Ellis: Patient | Nia: Pharmacy technician

ACCURACY CAN SOUND HELPFUL RATHER THAN OBSTRUCTIVE

A person who takes a medicine regularly may expect recognition to be enough. Acknowledge that familiarity and explain the exact unresolved detail. Give a pharmacist route instead of asking the patient to guess the prescription or treating the uncertainty as their fault.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which field is unclear?

- A A completed insurance decision
- C Confirmed pickup time

- B Strength
- D A verified recall lot

2. What does Ellis remember?

- A A confirmed pharmacist approval
- C A blue label and a usual prescription pattern

- B A guaranteed completion time
- D A verified current strength in the supplied facts

3. Who can take the clarification?

- A The blue label alone
- C Nia by selecting the most common strength

- B Pharmacist Maya
- D Another patient with a similar label

Answers and explanations: [page 85](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

prescription

An authorized order for a medicine under the applicable rules.

IN USE verify a prescription

intake

Receiving and organizing information needed to process an order.

IN USE complete prescription intake

prescriber

A professional authorized to prescribe the relevant medicine.

IN USE contact the prescriber

pharmacist

A qualified medicines professional with the relevant practice authority.

IN USE refer to the pharmacist

strength

The amount of active ingredient in a stated unit or quantity of the product.

IN USE clarify the strength

dose

The amount prescribed for a particular administration.

IN USE distinguish dose from strength

dosage form

The physical form of the product, such as a tablet or liquid.

IN USE verify the dosage form

active ingredient

The substance intended to produce the medicine's therapeutic effect.

IN USE identify the active ingredient

concentration

The amount of a substance within a specified quantity of a preparation.

IN USE verify the concentration

route

The specified way a medicine is taken or applied.

IN USE verify the route

frequency

How often a prescribed action or administration occurs.

IN USE clarify the frequency

directions for use

The authorized instructions for taking or using the medicine.

IN USE verify directions for use

Precision check

Strength and dose are not synonyms. A product may contain a particular amount per unit, while the prescribed administration uses a specified amount and schedule. This unit supplies neither and does not authorize calculation or selection from a remembered label.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

quantity

The amount of product specified for supply.

IN USE confirm the quantity

days supply

Days represented by quantity and verified directions under the relevant calculation rules; not proof of actual use.

IN USE verify days supply

brand name

A trade name for a particular product.

IN USE confirm the brand name

generic name

The recognized nonproprietary name of the medicine substance.

IN USE identify the generic name

product identifier

A code or reference identifying a specified product.

IN USE verify the product identifier

NDC

National Drug Code; a United States drug product identification system.

IN USE check the NDC

order field

A defined part of the order record.

IN USE clarify an order field

ambiguous entry

Information that permits more than one interpretation.

IN USE flag an ambiguous entry

clarification request

A request to resolve an unclear or missing instruction.

IN USE route a clarification request

current order

The order presently being processed, distinct from past records.

IN USE verify the current order

historical record

Information about a previous order or event.

IN USE consult the historical record

verified instruction

A direction confirmed through the appropriate process.

IN USE preserve the verified instruction

Precision check

Previous records may provide relevant context for the pharmacist, but they do not automatically resolve an unclear current order. Preserve what is uncertain and who supplied each detail. A familiar color, usual routine, or urgent request is not a verified instruction.

Phrases you can actually use

C Notice the language / 1 of 2

NAME THE UNRESOLVED FIELD

"The strength field on this order is unclear."

SEPARATE MEMORY AND CONFIRMATION

"The label color is useful context, but it does not confirm the current strength."

OFFER THE NEXT STEP

"Pharmacist Maya can take the clarification."

PRESERVE THE SOURCE

"I will record that you remember a blue label."

ACKNOWLEDGE FAMILIARITY

"I understand that you usually receive the same medicine."

STATE THE ROLE

"I cannot select a strength from an unclear entry."

AVOID BLAME

"We need the order clarified; I am not asking you to guess it."

DISTINGUISH TERMS

"Strength describes the product; dose concerns the prescribed amount used."

This order

Anchors the question to the current prescription rather than a general habit.

Useful context, but

Acknowledges the patient's contribution while preserving its evidential limit.

Cannot select

Names the specific prohibited action in this role.

Phrases you can actually use

C Notice the language / 2 of 2

CHECK THE CURRENT ORDER

"We need the current instructions confirmed, not just an earlier pattern."

AVOID A TIME PROMISE

"I do not yet have a confirmed completion time."

ASK ABOUT THE ACCEPTED HANDOFF

"Can Maya confirm receipt of this question?"

REPORT AN URGENT CONCERN

"Please tell the pharmacist about any concern that needs prompt attention."

ROUTE HISTORY APPROPRIATELY

"The pharmacist can review the relevant previous record."

KEEP STATUS ACCURATE

"The clarification is pending, not completed."

KEEP CLINICAL ADVICE SEPARATE

"Any question about how to take it should go to the pharmacist."

CLOSE WITH A CLEAR BOUNDARY

"We will use the verified instruction rather than fill the gap with a guess."

Not asking you to guess

Avoids shifting the uncertainty onto the patient.

Do not yet have

Preserves timing uncertainty without inventing a deadline.

Verified instruction

Identifies the required result of clarification, not the technician's preferred answer.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which reply is accurate?

- A** A blue label always identifies one strength.
- B** The most common strength must be correct.
- C** A previous order automatically overrides the current one.
- D** The strength is unclear; Maya can clarify it, and no completion time is confirmed.

2. Which distinction is correct?

- A** Strength describes product content per unit or quantity; dose describes the prescribed amount used.
- B** Strength and dose always mean the same thing.
- C** Label color is a universal product identifier.
- D** Days supply can be confirmed without relevant directions.

3. What should happen to the patient's recollection?

- A** Record it as reported context for the pharmacist, not verified current instructions.
- B** Copy the previous strength and mark the current field verified.
- C** Record the blue label as confirmation of product identity.
- D** Leave out the recollection so the pharmacist sees only the unclear field.

4. What must Nia avoid promising?

- A** A route to the available pharmacist
- B** An accurate description of the unclear field
- C** That the question remains pending
- D** A completion time that has not been confirmed

Answers and explanations: [page 85](#).

A familiar label does not resolve an unclear strength

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

intake / verified instruction / historical record / strength / dose / pharmacist / current order / clarification request / directions for use / ambiguous entry

- 01 **Ellis:** Nia, this should be the same prescription I usually collect. I remember the blue label, so can you finish it while I wait?
- 02 **Nia:** I understand, but the ¹ _____ field on this order is unclear. I cannot select it from the color or usual pattern; pharmacist Maya can take the clarification.
- 03 **Ellis:** I cannot remember the number. I could recognize the box, but I do not want to guess a strength and send you in the wrong direction.
- 04 **Nia:** You do not need to guess the ² _____. I will explain the specific missing detail and pass on what you remember as context, not ask you to choose between possible entries.
- 05 **Ellis:** The prescriber has ordered the same thing for a long time. Would looking at the previous record settle it?
- 06 **Nia:** A ³ _____ may help the pharmacist review the question, but it does not automatically replace unclear current instructions. We need the appropriate clarification rather than assume no change could have occurred.
- 07 **Ellis:** I sometimes mix up strength and dose. Are those two words referring to the same part of the prescription?
- 08 **Nia:** No. The ⁴ _____ is the prescribed amount used, while strength describes the product content per unit or quantity. I cannot determine either missing instruction from the label color.
- 09 **Ellis:** That explains why my description was not enough. I can tell Maya what I remember, but I cannot verify what the prescriber intended this time.
- 10 **Nia:** I will route a ⁵ _____ with that distinction intact. Your recollection can be included without being converted into a confirmed direction or a numerical value you did not provide.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

- 11 **Ellis:** Please make sure the question goes to Maya rather than just sit in a general note. I do not want to repeat everything at another counter.
- 12 **Nia:** I can give the ⁶ _____ the exact question and check that it is received. A handoff should identify who has the clarification, not merely say that someone will look at it.
- 13 **Ellis:** Can you tell me how long that will take? I need to decide whether to wait here or arrange another trip.
- 14 **Nia:** I do not have a confirmed completion time for this ⁷ _____. I can ask about the update route, but I should not promise a collection time before the relevant information is verified.
- 15 **Ellis:** Thanks. I have a bus to catch, so a guessed ten minutes would leave me in a difficult position.
- 16 **Nia:** Agreed. A ⁸ _____ is still needed. We will distinguish receiving the question, completing clarification, and completing the remaining pharmacy checks instead of treating them as one event.
- 17 **Ellis:** If I have a question about how to take the medicine once the order is clarified, should I ask Maya that as well?
- 18 **Nia:** Yes. Questions about the ⁹ _____ need the pharmacist's appropriate explanation. Resolving the strength field does not mean every question about your medicine has already been answered.
- 19 **Ellis:** Thank you. Please tell Maya that I remember a blue label and a usual pattern, but that I have not supplied a confirmed current strength.
- 20 **Nia:** I will preserve that in ¹⁰ _____. The strength remains unclear, Maya is the clarification route, and no completion time is confirmed. We will not fill the gap with an assumption.

ANSWERS / EXPLANATIONS: PAGE 86

1. strength | 2. current order | 3. historical record | 4. dose | 5. clarification request | 6. pharmacist | 7. ambiguous entry | 8. verified instruction | 9. directions for use | 10. intake

Clarify an unclear dosage form

F Say it

A current order has an unclear dosage-form field. The patient remembers a white box. Pharmacist Ren accepts the clarification. No product form or completion time has been confirmed.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

Ren / dosage form / unconfirmed / white box

Technician: "The unclear field concerns the ¹ _____."

Patient: "I remember a ² _____."

Technician: "The clarification was accepted by ³ _____."

Technician: "Completion timing remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 86

1. dosage form | 2. white box | 3. Ren | 4. unconfirmed

Rehearse with a fixed script

1. Read the strength question aloud without guessing a number or naming a medicine.
2. Complete the transfer with dosage form, the reported box description, the accepting pharmacist, and the actual timing status.
3. Check the four answers, then repeat the handoff while keeping memory separate from a verified current instruction.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Prior authorization required is not a final exclusion

A Read the situation

Explain an insurance response accurately and connect the patient with an accepted status query without promising coverage.

Patient Morgan sees the response "prior authorization required" for a pharmacy claim and believes the medicine is permanently excluded. No authorization decision or accepted claim has arrived. Technician Alex can explain the recorded status without promising coverage, a price, or a deadline. Billing colleague Inez has accepted the status query and can explain the request route. The conversation should distinguish the plan requirement, any later decision, and the current pharmacy status. No particular insurance policy, appeal outcome, clinical alternative, or patient payment amount is established in the scenario.

PEOPLE / Morgan: Patient | Alex: Pharmacy technician

TRANSLATE THE MESSAGE WITHOUT SOFTENING ITS MEANING

A short insurer response can sound like a permanent refusal. Explain the actual status in ordinary language while preserving what remains unknown. Avoid both a hopeless conclusion and reassurance that approval is merely a formality.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the current response say?

- A Permanent exclusion confirmed
- C No further plan process exists

- B Claim accepted and paid
- D Prior authorization required

2. What is still absent?

- A An authorization decision and an accepted claim
- C An accepted status-query owner

- B Any recorded response
- D A patient question

3. Who accepted the status query?

- A The patient as plan decision-maker
- C An unnamed courier

- B The wholesaler
- D Inez

Answers and explanations: page 87.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

prior authorization

Plan approval that may be required before specified care or a prescription is covered.

IN USE request prior authorization

health plan

The insurance arrangement with defined benefits and rules.

IN USE verify the health plan

claim

A request submitted for payment or benefit processing.

IN USE submit a claim

claim response

The result or message returned for a submitted claim.

IN USE explain the claim response

adjudication

The process of evaluating a claim under the applicable plan rules.

IN USE check claim adjudication

response code

A coded message identifying a claim-processing result or requirement.

IN USE clarify the response code

rejected claim

A claim not accepted as submitted under the returned response.

IN USE investigate a rejected claim

accepted claim

A claim accepted under the relevant processing response.

IN USE verify an accepted claim

coverage decision

The plan's determination about the relevant coverage request.

IN USE confirm the coverage decision

coverage exclusion

A benefit or service not covered under the applicable plan terms.

IN USE verify a coverage exclusion

formulary

A plan's list of medicines and their coverage-related conditions.

IN USE check the formulary

benefit tier

A plan category that may affect coverage conditions and cost sharing.

IN USE identify the benefit tier

Precision check

Prior authorization required states a plan requirement. It does not by itself establish permanent exclusion, approval, an accepted claim, or the final patient price. Explain the actual response and have the appropriate colleague clarify the applicable request route.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

copayment

A fixed patient payment for a covered service under the plan.

IN USE verify the copayment

coinsurance

A patient share calculated as a percentage under the applicable benefit.

IN USE explain coinsurance

deductible

The amount a person may need to pay before specified plan benefits apply.

IN USE verify the deductible

out-of-pocket cost

The amount the person pays rather than the plan.

IN USE confirm out-of-pocket cost

pharmacy network

Pharmacies participating under a plan's arrangements.

IN USE verify pharmacy network status

PBM

Pharmacy benefit manager; an organization administering defined pharmacy-benefit functions.

IN USE identify the relevant PBM

member identifier

The plan reference identifying the insured member.

IN USE verify the member identifier

step therapy

A plan requirement to try specified treatment steps under its rules.

IN USE clarify a step-therapy requirement

quantity limit

A plan or order restriction on the amount supplied under specified conditions.

IN USE verify a quantity limit

exception request

A request for an exception under the plan's defined process.

IN USE route an exception request

appeal

A request to review a relevant adverse decision through the applicable process.

IN USE clarify the appeal route

status query

A request to establish the current stage, requirement, or result.

IN USE accept a status query

Precision check

A coverage decision and claim processing can involve different information and stages. Do not promise that one status automatically resolves every payment condition. Plan rules differ; use the actual response and verified benefit information rather than a rule remembered from another patient.

Phrases you can actually use

C Notice the language / 1 of 2

READ THE STATUS

"The response says prior authorization required."

CORRECT AN INFERENCE

"That message alone does not establish a permanent exclusion."

NAME WHAT IS MISSING

"We do not yet have an authorization decision or an accepted claim."

OFFER THE ROUTE

"Inez can explain how the request is handled for this plan."

EXPLAIN THE TERM

"The plan is asking for its approval process to be completed."

AVOID FALSE REASSURANCE

"It also does not mean approval is guaranteed."

IDENTIFY THE OWNER

"Inez has accepted the status query."

KEEP THE PRICE UNCONFIRMED

"I cannot confirm your final payment from this response alone."

That message alone

Limits the conclusion to what the available response establishes.

Also does not mean

Corrects overoptimism as well as an overly negative inference.

Has accepted

Names a real handoff owner rather than a vague future referral.

Phrases you can actually use

C Notice the language / 2 of 2

SEPARATE STAGES

"A request sent is not a decision received."

CLARIFY RESPONSIBILITY

"Who is providing the information the plan needs?"

KEEP DEADLINES HONEST

"No response deadline has been confirmed in this conversation."

CHECK UNDERSTANDING

"The requirement is identified, but the outcome remains open."

PRESERVE THE ORIGINAL RESPONSE

"We should keep the exact message in the query."

AVOID A CLINICAL SUBSTITUTION

"Any medicine-alternative question needs the pharmacist."

DO NOT PRE-JUDGE AN APPEAL

"The relevant next process depends on the actual decision and plan rules."

CLOSE WITH A USEFUL NEXT STEP

"I will connect your question with Inez and the verified request route."

From this response alone

Explains why a final price cannot yet be promised.

Request sent versus decision received

Separates activity from outcome.

Depends on the actual decision

Avoids inventing the right appeal or exception process before the facts are known.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which explanation is accurate?

- A** The pharmacy can ignore the response code.
- B** The plan requires prior authorization; no decision or accepted claim has arrived.
- C** The medicine is excluded forever.
- D** Approval is certain once someone submits a form.

2. Which statement overstates the billing handoff?

- A** No final patient price is established.
- B** Inez accepted the query, so the claim is now approved.
- C** Inez can explain the request route.
- D** The original response should be preserved.

3. Which statement accurately separates the request from the result?

- A** Submission does not establish that authorization was granted.
- B** Submission confirms approval unless the plan sends a refusal that day.
- C** Submission confirms the patient's final copayment.
- D** Submission changes the pharmacy claim to accepted automatically.

4. What should happen to a question about a different medicine?

- A** Choose one solely because another plan covered it.
- B** Let the claim code prescribe a replacement.
- C** Promise that every cheaper medicine is equivalent.
- D** Refer it to the pharmacist for appropriate clinical discussion.

Answers and explanations: page 87.

Prior authorization required is not a final exclusion

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

status query / copayment / appeal / pharmacist / out-of-pocket cost / prior authorization / claim response / accepted claim / health plan / coverage decision

- 01 **Morgan:** Alex, this message says prior authorization required. Does that mean my medicine is excluded permanently and there is no point asking the plan anything else?
- 02 **Alex:** It identifies a ¹ _____ requirement, not a permanent exclusion by itself. We do not yet have the plan's authorization decision or an accepted claim, so the outcome remains open.
- 03 **Morgan:** My friend said you send a form and it goes through. Is that certain, or are we waiting for a real decision?
- 04 **Alex:** I cannot promise that ² _____. The request has to follow the actual plan process, and the result is not established merely because someone starts gathering or sending information.
- 05 **Morgan:** So it is neither a final no nor a definite yes. What is actually blocking the claim today?
- 06 **Alex:** We should preserve the exact ³ _____ and explain its current meaning. It tells us what requirement was returned, but it does not confirm the final price, supply status, or a favorable decision.
- 07 **Morgan:** Who is looking into that requirement? I do not want my question to be passed from person to person without anyone knowing what happens next.
- 08 **Alex:** Inez has accepted the ⁴ _____. She can explain the request route for this plan and identify the information needed, rather than leave you with a general instruction to contact somebody.
- 09 **Morgan:** If Inez speaks with the plan, will that automatically make the claim accepted, or could more information still be needed afterward?
- 10 **Alex:** An ⁵ _____ is a separate status that needs verification. A telephone conversation or a submitted request does not, by itself, show that the claim has been accepted.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

- 11 **Morgan:** I also need to know what I might pay. Can you read the current message and tell me my final amount now?
- 12 **Alex:** This response does not establish your ⁶ _____. The applicable benefit information and claim result need to be checked, and I should not supply a number that the record does not support.
- 13 **Morgan:** My friend has the same medicine on a different plan and pays a small fixed amount. Does that tell us what my plan will do?
- 14 **Alex:** Their ⁷ _____ does not establish yours. Plans and individual benefit circumstances can differ, so we need your actual plan information rather than copy another person's experience.
- 15 **Morgan:** Should I immediately appeal, then, or is that a different step from the requirement we are looking at now?
- 16 **Alex:** An ⁸ _____ depends on the relevant decision and process. Inez can clarify the current route; I should not skip to an appeal or promise a particular outcome before that is established.
- 17 **Morgan:** If another medicine might be an option, I would like that question answered too. I do not want to make a medical decision from an insurance message.
- 18 **Alex:** The ⁹ _____ should discuss any clinical alternative with you through the appropriate process. We can keep that question separate from Inez's work on the claim requirement.
- 19 **Morgan:** Please tell Inez that I understand the requirement is not a permanent exclusion, but I still need the route, outstanding information, and actual next step.
- 20 **Alex:** I will. The ¹⁰ _____ process remains unresolved, no final charge is confirmed, and Inez owns the status query. We will explain verified developments without turning them into a guarantee.

ANSWERS / EXPLANATIONS: PAGE 88

1. prior authorization | 2. coverage decision | 3. claim response | 4. status query | 5. accepted claim | 6. out-of-pocket cost | 7. copayment | 8. appeal | 9. pharmacist | 10. health plan

Explain a different unresolved claim requirement

F Say it

A response says quantity limit exceeded. No exception decision or accepted claim is available. Billing colleague Sam accepts the query; the pharmacist handles questions about changing treatment.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

unconfirmed / pharmacist / quantity limit / Sam

Technician: "The returned requirement concerns a ¹ _____."

Patient: "The billing query is with ² _____."

Technician: "Any clinical change should be discussed with the ³ _____."

Technician: "The exception outcome remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 88

1. quantity limit | 2. Sam | 3. pharmacist | 4. unconfirmed

Rehearse with a fixed script

1. Read the prior-authorization exchange with requirement, decision, and accepted claim clearly separated.
2. Complete the quantity-limit transfer using its named billing colleague and pharmacist route.
3. Check the key, then repeat the final status without promising either a payment amount or an exception decision.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A status call is not a promise of supply

A Read the situation

03

Explain an unresolved renewal and make a reliable communication commitment without promising prescriber response or medicine availability.

At 11:00, patient Casey asks technician Jo about a refill. The record shows no refills remaining. A renewal request was sent to the prescriber at 09:00, but no response has arrived. Jo can call Casey between 15:00 and 16:00 with a status update. No prescriber response time, supply, or collection time is confirmed. Jo needs to explain what was sent, what remains pending, and what the call will cover. Any concern about running out or interrupted treatment should be referred promptly to the pharmacist rather than treated as safe to wait for the update.

PEOPLE / Casey: Patient | Jo: Pharmacy technician

GIVE CERTAINTY ABOUT THE ACTION YOU CONTROL

Patients often hear we will call as it will be ready. Name the purpose of the call and repeat the distinction without sounding evasive. A clear update commitment can be useful even when the pharmacy cannot promise another professional's response.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the refill record show?

A A confirmed pickup time

C No refills remaining

B A completed prescriber response

D An approved new supply

2. When was the renewal request sent?

A 15:00

C 09:00

B 16:00

D 11:00

3. What can Jo commit to?

A A prescriber answer at 16:00

C A status call between 15:00 and 16:00

B A clinical instruction to wait regardless of need

D Guaranteed medicine supply at 15:00

Answers and explanations: [page 89](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

refill

A further supply under an existing prescription when permitted and available.

IN USE request a refill

refills remaining

The number of further supplies recorded as authorized under the prescription.

IN USE check refills remaining

renewal

A request or authorization for a further prescription when the existing authorization is insufficient.

IN USE request a prescription renewal

renewal request

A message asking the prescriber to authorize the relevant further prescription.

IN USE send a renewal request

prescriber response

The actual reply from the prescribing professional or practice.

IN USE await the prescriber response

authorization pending

A status in which the required approval has not yet been received.

IN USE explain authorization pending

request timestamp

The time recorded for sending or receiving the request.

IN USE verify the request timestamp

transmission confirmation

Evidence that a message was sent or delivered through a system, not that it was approved.

IN USE check transmission confirmation

response window

A stated period for an expected or promised response whose status must be clear.

IN USE qualify the response window

callback window

The agreed period in which a return call will be made.

IN USE confirm the callback window

status call

A call providing current information rather than guaranteeing an outcome.

IN USE make a status call

collection time

The time at which an order is confirmed available for pickup.

IN USE confirm collection time

Precision check

A transmission confirmation may show that the request left the system or reached a destination. It is not a prescriber approval. Similarly, a status call between 15:00 and 16:00 is a communication commitment, not a guaranteed collection window.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

supply authorization

The permission needed for the particular medicine supply under applicable rules.

IN USE verify supply authorization

partial fill

Supply of less than the prescribed quantity where permitted under applicable arrangements.

IN USE refer a partial-fill question

emergency supply

A medicine supply considered under specific legal and clinical requirements.

IN USE refer an emergency-supply question

prescription transfer

Movement of prescription information between pharmacies where permitted.

IN USE clarify prescription-transfer requirements

refill too soon

A processing response indicating that the requested supply is earlier than the applicable rules allow.

IN USE explain a refill-too-soon response

remaining quantity

The amount still available or authorized under the relevant record.

IN USE verify the remaining quantity

adherence

Taking medicine in accordance with the agreed clinical instructions.

IN USE discuss an adherence concern

treatment interruption

A break in the prescribed treatment that may require clinical advice.

IN USE report a treatment interruption

continuity of therapy

Maintaining appropriate treatment under qualified guidance.

IN USE support continuity of therapy

follow-up owner

The person responsible for the specified next contact or action.

IN USE name the follow-up owner

contact outcome

Whether and how a communication attempt reached the intended person.

IN USE record the contact outcome

pending response

A reply still awaited rather than received.

IN USE track a pending response

Precision check

No refills remaining does not authorize the technician to invent a supply or advise the person to interrupt treatment. Questions about immediate need, emergency supply, partial fills, or alternatives require the pharmacist and applicable rules; the examples confer no entitlement.

Phrases you can actually use

C Notice the language / 1 of 2

03

STATE THE RECORD

"The record shows no refills remaining."

STATE THE CURRENT TIME

"As of eleven, no response has arrived."

OWN THE FOLLOW-UP

"I will call you with the status between three and four."

AVOID A PRESCRIBER PROMISE

"I cannot guarantee when the prescriber will respond."

GIVE THE SENT TIME

"We sent the renewal request at nine this morning."

KEEP STAGES SEPARATE

"Sending the request does not mean the renewal is approved."

EXPLAIN THE CALL

"That is an update window, not a confirmed collection time."

KEEP SUPPLY UNCONFIRMED

"No further supply has been confirmed yet."

As of eleven

Establishes the time boundary of the reported status.

Does not mean approved

Separates transmission from authorization.

With the status

Specifies the call's purpose instead of implying readiness.

Phrases you can actually use

C Notice the language / 2 of 2

03

ROUTE IMMEDIATE NEED

"Please discuss any concern about running out with the pharmacist now."

CLARIFY THE UPDATE CONTENT

"I will tell you whether a response has arrived and what remains pending."

VERIFY CONTACT

"Let us confirm the appropriate contact details."

IDENTIFY THE NEXT OWNER

"I am responsible for the agreed status call."

DO NOT ADVISE INTERRUPTION

"I cannot tell you to stop or change treatment while waiting."

PRESERVE AN UNSUCCESSFUL ATTEMPT

"If I cannot reach you, I will record the attempt through our process."

AVOID ASSUMING DENIAL

"No response is not the same as a refusal."

CLOSE ACCURATELY

"The request is pending, and I will update you even if it is still unresolved."

Even if unresolved

Makes the update commitment useful without guaranteeing an external result.

No response versus refusal

Distinguishes silence from a negative decision.

Now

Keeps a current clinical need separate from a later routine update.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which update is accurate at 11:00?

- A** The 09:00 renewal request is unanswered; Jo will give a status update between 15:00 and 16:00.
- B** The renewal is approved because two hours have passed.
- C** Pickup is guaranteed at 15:00.
- D** No response proves permanent refusal.

2. What does a successful transmission not prove?

- A** That a system recorded transmission
- B** A reason to retain the request timestamp
- C** The need to check for an actual reply
- D** Prescriber authorization

3. What should happen to a concern about running out?

- A** Recommend borrowing another person's medicine.
- B** Refer it promptly to the pharmacist for appropriate assessment.
- C** Tell the patient to wait regardless until 16:00.
- D** Promise an emergency supply automatically.

4. Who owns the promised call?

- A** Jo
- B** The patient automatically
- C** An unidentified prescriber receptionist
- D** The wholesaler

Answers and explanations: [page 89](#).

A status call is not a promise of supply

E Conversations / turns 1-10

03

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

emergency supply / treatment interruption / status call / refills remaining / request timestamp / callback window / Transmission confirmation / follow-up owner / pending response / contact outcome

- 01 **Casey:** Jo, I asked for a refill earlier. It is eleven now, and I would like to know whether I can come and collect it this afternoon.
- 02 **Jo:** The record shows no ¹ _____. We sent a renewal request at nine, but no response has arrived as of eleven, so I cannot confirm collection yet.
- 03 **Casey:** Does that mean the prescriber refused it? I am worried that no reply means they have decided I should not have it anymore.
- 04 **Jo:** A ² _____ is not the same as a refusal. I can report what the record shows, but I should not invent a decision or a reason for the delay.
- 05 **Casey:** Can you check that it actually went out? Last time, requested meant someone had put it on a list to send later.
- 06 **Jo:** The ³ _____ is nine this morning. We should distinguish that sent event from a task waiting to be sent, while also keeping it separate from a completed authorization.
- 07 **Casey:** If the system says the message went through, why can that not count as approval? It seems as though the practice must have received it.
- 08 **Jo:** ⁴ _____ concerns the message, not the decision. Even when delivery is confirmed, the actual response and any remaining pharmacy checks still need to be established.
- 09 **Casey:** What can you tell me about this afternoon? I do not want to travel here on the assumption that a phone call means the order is ready.
- 10 **Jo:** I can promise a ⁵ _____ between three and four. I will tell you whether a response has arrived and what remains pending; that is not a guaranteed collection time.

The conversation, continued

E Conversations / turns 11-20

03

Continue with the word bank on page 28.

- 11 **Casey:** So if there is still no answer by then, you will call anyway rather than wait silently until the order is eventually finished?
- 12 **Jo:** Yes. I own that ⁶ _____. The update should happen within it even if the status is unchanged, so you know what has and has not been confirmed.
- 13 **Casey:** I am worried about running out before anyone replies. Can I speak to the pharmacist now, rather than wait for the afternoon call?
- 14 **Jo:** Please discuss that possible ⁷ _____ with the pharmacist now. I cannot advise you to stop, change, or wait without the appropriate clinical guidance.
- 15 **Casey:** Could the pharmacist discuss whether any immediate supply route is possible? I understand you cannot promise one just because I ask.
- 16 **Jo:** The pharmacist can assess the actual need and rules around any ⁸ _____ or other appropriate option. I cannot authorize that independently or guarantee that a particular route applies.
- 17 **Casey:** Please use my verified contact details for the update. If you cannot reach me, I would not want the note to say I was fully informed.
- 18 **Jo:** I will record the actual ⁹ _____. A call attempt is not the same as speaking with you, and any further action must follow our communication process.
- 19 **Casey:** Thank you. I understand the renewal is still pending, no supply time is confirmed, and you will update me between three and four.
- 20 **Jo:** Correct. I am the ¹⁰ _____ for that call. We will keep your immediate concern with the pharmacist and keep the later update separate from any promise of prescriber response or medicine availability.

ANSWERS / EXPLANATIONS: PAGE 90

1. refills remaining | 2. pending response | 3. request timestamp | 4. Transmission confirmation | 5. status call | 6. callback window | 7. treatment interruption | 8. emergency supply | 9. contact outcome | 10. follow-up owner

Explain a later renewal update window

F Say it

No refills remain. A request sent at 10:30 has no reply at 12:30. Technician Mei owns a status call between 16:00 and 17:00. No collection time is confirmed.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

16:00 and 17:00 / 10:30 / no reply / unconfirmed

Technician: "The request was sent at ¹ _____."

Patient: "The current status at 12:30 is ² _____."

Technician: "Mei will call between ³ _____."

Patient: "The collection time remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 90

1. 10:30 | 2. no reply | 3. 16:00 and 17:00 | 4. unconfirmed

Rehearse with a fixed script

1. Read the renewal call with nine, eleven, and the three-to-four update window clearly distinguished.
2. Repeat the transfer using 10:30, 12:30, and 16:00 to 17:00; do not turn its call into a collection promise.
3. Check all four answers, then repeat the prompt pharmacist referral for a concern about running out.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Wednesday is an estimate, not a collection promise

A Read the situation

Explain a stock shortage, qualify a supplier forecast, and route alternative-treatment questions without choosing a substitute.

The pharmacy has no stock for patient Robin's order. The wholesaler estimates delivery on Wednesday, but shipment has not been confirmed. Technician Lee can check the next supplier update on Tuesday at 16:00. The pharmacist handles questions about alternatives or immediate clinical need. Lee must separate no stock now, a future delivery estimate, dispatch confirmation, actual receipt, and readiness for collection. No exact arrival hour, guaranteed quantity, substitute medicine, or patient collection time is established. A forecast must not become a clinical instruction to wait.

PEOPLE / Robin: Patient | Lee: Pharmacy technician

A USEFUL ESTIMATE INCLUDES ITS SOURCE AND LIMITS

A patient may turn Wednesday into I can collect Wednesday morning. Keep the timing qualification attached each time it is repeated. Explain what will be checked and where the patient can raise an immediate need instead of giving vague reassurance.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is the current local stock position?

- A A verified substitute already selected
- C No stock for this order

- B A shipment already received
- D A confirmed full quantity ready for collection

2. What does Wednesday describe?

- A The wholesaler's unconfirmed delivery estimate
- C A completed pharmacist review

- B A guaranteed morning collection time
- D A prescribed treatment interruption

3. When can Lee check the supplier update?

- A Tuesday at 16:00
- C Immediately after a confirmed receipt already recorded

- B Wednesday at 09:00 guaranteed
- D Monday at noon

Answers and explanations: page 91.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

stock on hand

Product physically held at the location under the relevant inventory record.

IN USE verify stock on hand

available stock

Stock actually available for the intended use after relevant restrictions and checks.

IN USE confirm available stock

stockout

A situation in which the required product is not available in stock.

IN USE report a stockout

backorder

An order awaiting stock rather than immediately fulfilled.

IN USE track a backorder

wholesaler

A business supplying products to pharmacies or other authorized customers.

IN USE contact the wholesaler

supplier estimate

A forecast given by the supplying organization.

IN USE qualify the supplier estimate

shipment confirmation

Verification that the relevant shipment has actually been arranged or sent as specified.

IN USE obtain shipment confirmation

dispatch

Sending goods from the supplier or distribution point.

IN USE confirm dispatch

carrier tracking

Recorded information about movement of a shipment.

IN USE check carrier tracking

receipt

Actual arrival of the relevant goods at the destination.

IN USE confirm receipt

delivery window

The period in which a delivery is expected or committed, with status specified.

IN USE clarify the delivery window

allocation

A quantity assigned to a particular order or customer.

IN USE verify the allocation

Precision check

No stock at one pharmacy does not by itself prove a regional or national shortage. Likewise, an expected delivery day does not confirm dispatch, the full requested quantity, pharmacy receipt, or collection readiness. Name the status actually known.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

lead time

The period between an order or request and the relevant delivery or completion.

IN USE qualify the lead time

supply disruption

An interruption affecting the availability or movement of a product.

IN USE report a supply disruption

regional shortage

Limited availability across a defined area rather than one pharmacy alone.

IN USE verify a regional shortage

product availability

Whether a specified product can actually be obtained under the relevant conditions.

IN USE confirm product availability

generic substitution

Supply of an eligible equivalent generic product under applicable rules and authority.

IN USE refer a generic-substitution question

therapeutic alternative

A different treatment option requiring appropriate clinical consideration.

IN USE discuss a therapeutic alternative

interchangeability

Whether products may replace one another under relevant clinical and legal requirements.

IN USE verify interchangeability

order allocation

The part of stock or supply assigned to the particular order.

IN USE check order allocation

supplier checkpoint

An agreed time to obtain updated supplier information.

IN USE confirm the supplier checkpoint

collection readiness

Confirmation that the order has completed the relevant steps for pickup.

IN USE verify collection readiness

partial delivery

Receipt of less than the ordered quantity.

IN USE identify a partial delivery

unconfirmed forecast

An expected outcome that has not been established as a commitment or event.

IN USE label an unconfirmed forecast

Precision check

A possible alternative is not automatically interchangeable with the prescribed product. Route clinical and substitution questions to the pharmacist under actual rules. The supplier checkpoint is a communication step, not a recommendation about whether the person can safely wait.

Phrases you can actually use

C Notice the language / 1 of 2

STATE CURRENT AVAILABILITY

"We do not currently have stock for your order."

QUALIFY DISPATCH

"Shipment has not yet been confirmed."

NAME THE CHECKPOINT

"I can check the supplier update Tuesday at sixteen hundred."

AVOID OVERSTATING SHORTAGE

"This confirms our local stock position, not availability everywhere."

ATTRIBUTE THE ESTIMATE

"The wholesaler estimates Wednesday delivery."

AVOID A COLLECTION PROMISE

"I cannot confirm a pickup time from that estimate alone."

SEPARATE STAGES

"Dispatch, receipt, and readiness for collection are different statuses."

CHECK QUANTITY

"We need the quantity for this order confirmed as well as timing."

Currently

Describes the present stock position without making it permanent.

The wholesaler estimates

Identifies the source and forecast status.

From that estimate alone

Explains why collection timing remains uncertain.

Phrases you can actually use

C Notice the language / 2 of 2

ROUTE ALTERNATIVES

"The pharmacist can discuss appropriate alternatives with you."

ADDRESS IMMEDIATE NEED

"Please tell the pharmacist about any concern that needs prompt attention."

REQUEST A USEFUL SUPPLIER UPDATE

"Has this order been allocated and dispatched, or is it still awaiting confirmation?"

EXPLAIN REMAINING WORK

"Receipt still needs the relevant pharmacy checks before collection is confirmed."

DO NOT SUBSTITUTE INDEPENDENTLY

"I cannot choose another medicine from the stock shelf."

PRESERVE THE UNCERTAINTY

"Wednesday remains an estimate, not a guarantee."

KEEP RECEIPT FACTUAL

"A tracking movement does not mean the pharmacy has received the product."

CLOSE THE MESSAGE

"No stock now; Wednesday estimated; shipment unconfirmed; supplier check Tuesday at four."

For this order

Prevents a general supply update from being mistaken for allocated stock.

Appropriate alternatives

Leaves clinical suitability and legal substitution with the pharmacist.

Still needs

Makes later required steps visible without inventing their completion time.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which statement preserves the facts?

- A** We have no stock; Wednesday is estimated, shipment is unconfirmed, and Lee will check Tuesday at 16:00.
- B** Wednesday morning collection is guaranteed.
- C** A supplier forecast means the order has arrived.
- D** A local stockout proves every pharmacy is out.

2. What does carrier movement not establish by itself?

- A** That a carrier event was recorded
- B** A reason to check the specific order
- C** Possible progress in transport
- D** Collection readiness at the pharmacy

3. Who should address an alternative medicine question?

- A** Lee by selecting a similar-looking box
- B** The carrier driver
- C** The patient by copying a neighbor's medicine
- D** The pharmacist under the applicable process

4. What does Tuesday at 16:00 represent?

- A** An automatic approval to change treatment
- B** A promised Wednesday arrival hour
- C** A supplier information check, not guaranteed supply or clinical advice to wait
- D** A confirmed delivery to the patient

Answers and explanations: [page 91](#).

Wednesday is an estimate, not a collection promise

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

regional shortage / supplier estimate / Collection readiness / Carrier tracking / therapeutic alternative / shipment confirmation / stockout / supplier checkpoint / order allocation / partial delivery

- 01 **Robin:** Lee, I was told there was no stock for my order. Has that changed, or are you still waiting for a delivery?
- 02 **Lee:** We still have a ¹ _____ for this order. The wholesaler estimates Wednesday delivery, but shipment is not confirmed, so I cannot tell you the order is available now.
- 03 **Robin:** I can get a lift Wednesday morning. Should I book it, or does Wednesday just mean that is when the supplier hopes to deliver?
- 04 **Lee:** No. It is a ² _____, without a confirmed hour or shipment. I should not turn that into a Wednesday-morning collection promise when the information does not establish one.
- 05 **Robin:** Does no shipment confirmation mean the supplier definitely cannot provide it, or only that you do not yet have confirmation?
- 06 **Lee:** We do not yet have ³ _____. That is different from a confirmed cancellation. I will keep the uncertainty clear rather than replace it with either a guarantee or an unsupported refusal.
- 07 **Robin:** When will you next be able to check? I do not want to call repeatedly if there is a particular update point.
- 08 **Lee:** The ⁴ _____ is Tuesday at sixteen hundred. I can check the latest status then, including whether the order has been allocated, dispatched, or remains unconfirmed.
- 09 **Robin:** If the tracking page starts showing movement, would that be enough for you to tell me I can collect it?
- 10 **Lee:** ⁵ _____ may show progress in transit, but it is not the same as pharmacy receipt. The actual product and order status still need to be verified.

The conversation, continued

E Conversations / turns 11-20

04

Continue with the word bank on page 37.

- 11 **Robin:** Is my whole quantity included? Last time a delivery arrived, but only part of what I needed was in it.
- 12 **Lee:** Correct. We need the ⁶ _____ confirmed for your order. A shipment to the pharmacy does not automatically prove that the requested quantity is included or complete.
- 13 **Robin:** If only some arrives, I suppose that is another question rather than a reason to tell me the entire order has been fulfilled.
- 14 **Lee:** Yes. A ⁷ _____ should be described accurately. Any decision about what may be supplied to you must follow the pharmacist's process and applicable requirements, not my assumption.
- 15 **Robin:** What about a different medicine? I would like to discuss options if waiting is a problem, but I do not want a substitute chosen just because it is on the shelf.
- 16 **Lee:** The pharmacist can discuss a ⁸ _____ or other appropriate route. Please raise any immediate need with them now; the delivery estimate is not advice that you should interrupt treatment or wait without guidance.
- 17 **Robin:** Does your stock problem mean the product is unavailable across the region? That would affect how I understand the situation.
- 18 **Lee:** We have not established a ⁹ _____. The fact I can confirm is our current local stock position. A broader claim needs its own verified information.
- 19 **Robin:** Thank you. I will not treat Wednesday as a pickup appointment. Please keep the supplier's estimate and any later confirmed facts separate.
- 20 **Lee:** I will. ¹⁰ _____ comes after the appropriate steps are confirmed. For now: no stock, Wednesday estimated, shipment unconfirmed, and the supplier checkpoint Tuesday at four.

ANSWERS / EXPLANATIONS: PAGE 92

1. stockout | 2. supplier estimate | 3. shipment confirmation | 4. supplier checkpoint | 5. Carrier tracking | 6. order allocation | 7. partial delivery | 8. therapeutic alternative | 9. regional shortage | 10. Collection readiness

Explain a Friday estimate without promising pickup

F Say it

No stock is available for an order. The supplier estimates Friday delivery, but dispatch is unconfirmed. Technician Bea can check Thursday at 11:00. The pharmacist handles alternative-treatment questions.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

unconfirmed / Thursday at 11:00 / pharmacist / Friday

Technician: "Estimated delivery is ¹ _____."

Patient: "Dispatch is still ² _____."

Technician: "The supplier check is ³ _____."

Patient: "An alternative-treatment question goes to the ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 92

1. Friday | 2. unconfirmed | 3. Thursday at 11:00 | 4. pharmacist

Rehearse with a fixed script

1. Read the supplier-status exchange aloud, stressing estimated rather than guaranteed.
2. Complete the Friday transfer with Thursday at 11:00 as the information checkpoint.
3. Check the key, then repeat the distinction between dispatch, receipt, and collection readiness.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The label question has a named pharmacist

A Read the situation

Recognize an auxiliary-label question and complete a clear pharmacist handoff without interpreting or changing the instruction.

At pickup, patient Arden asks technician Kit what an auxiliary label means. Kit can identify the label but is not authorized to interpret its instruction. Pharmacist Ravi is available at the consultation counter and has accepted the question. No instructions are changed in this scenario, and the actual label wording is not supplied as a clinical direction. Kit should acknowledge the uncertainty, explain the handoff, and help Arden reach Ravi without implying that a label is optional or that handing over the package has already answered the question.

PEOPLE / Arden: Patient | Kit: Pharmacy technician

A QUESTION AT PICKUP IS PART OF THE VISIT

A queue can make someone reluctant to ask about medicine instructions. Acknowledge the question without rushing the person or suggesting that familiar wording is obvious. A named pharmacist handoff is more helpful than pointing vaguely toward another counter.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does Arden ask about?

- A A confirmed recall lot
- C A final insurer appeal

- B The wholesaler's route
- D An auxiliary label

2. Who has accepted the question?

- A The next patient in line
- C Pharmacist Ravi

- B An unidentified prescriber
- D Kit as independent clinical interpreter

3. What changes to the instructions are established?

- A An automatic change in frequency
- C A new dose chosen by Kit

- B None
- D Removal of every warning

Answers and explanations: [page 93](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

auxiliary label

An additional label providing a relevant medicine instruction or caution.

IN USE identify the auxiliary label

dispensing label

The pharmacy label with the relevant patient and medicine directions.

IN USE verify the dispensing label

patient counseling

A professional discussion helping the patient understand the medicine and its use.

IN USE arrange patient counseling

consultation counter

The designated place for the pharmacist discussion.

IN USE direct to the consultation counter

medication guide

Patient information supplied for certain medicines under relevant requirements.

IN USE provide the medication guide

patient information leaflet

Written product information intended for the patient.

IN USE identify the patient information leaflet

package insert

Product information supplied with the medicine, with audience and purpose varying by format.

IN USE identify the package insert

warning

Information drawing attention to a relevant risk or required caution.

IN USE clarify a warning

precaution

A condition or measure requiring care in using a medicine.

IN USE ask about a precaution

contraindication

A condition in which a medicine or treatment should not be used under its labeling or clinical assessment.

IN USE refer a contraindication question

interaction

A relevant effect arising from a medicine used with another substance or condition.

IN USE ask about an interaction

food interaction

A possible effect involving medicine use and food.

IN USE refer a food-interaction question

Precision check

An auxiliary label adds information; it is not optional merely because it is separate from the main label. The technician can identify which label prompted the question while the pharmacist explains its meaning for the actual medicine and patient.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

alcohol interaction

A possible effect involving medicine use and alcohol.

IN USE refer an alcohol-interaction question

drowsiness

A tendency to feel sleepy or less alert.

IN USE report drowsiness

storage condition

A requirement for keeping a product appropriately.

IN USE clarify storage conditions

refrigeration

Storage under specified refrigerated conditions when required.

IN USE clarify a refrigeration instruction

light protection

Storage or handling intended to limit relevant light exposure.

IN USE clarify light-protection wording

measuring device

A device intended to measure an appropriate quantity accurately.

IN USE identify the supplied measuring device

administration device

Equipment used to deliver a medicine under the specified directions.

IN USE identify the administration device

privacy

Protection of personal information and conversation from inappropriate exposure.

IN USE protect privacy

teach-back

A check in which the person explains their understanding so the professional can clarify it.

IN USE use teach-back appropriately

accepted handoff

A transfer of a question or task acknowledged by the receiving person.

IN USE confirm an accepted handoff

instruction interpretation

Explanation of what a direction means for the person's medicine use.

IN USE refer instruction interpretation

counseling completion

Confirmation that the actual professional discussion occurred, not merely that it was offered.

IN USE record counseling completion

Precision check

Offered counseling, accepted handoff, and completed counseling describe different events. Ravi accepting the question gives it an owner. It does not prove that the discussion has happened, that the person understands, or that any instruction has changed.

Phrases you can actually use

C Notice the language / 1 of 2

WELCOME THE QUESTION

"Thank you for asking about that label."

STATE THE BOUNDARY

"I can identify it, but I cannot interpret the instruction for you."

GIVE A CLEAR LOCATION

"Ravi is available at the consultation counter."

AVOID AN OPTIONALITY CLAIM

"An additional label is not automatically optional."

IDENTIFY THE ITEM

"You are referring to the auxiliary label beside the main dispensing label."

NAME THE PHARMACIST

"Ravi has accepted your question."

KEEP THE WORDING INTACT

"I will pass on the exact label and your question."

DO NOT CHANGE INSTRUCTIONS

"I am not changing any direction on this package."

Thank you for asking

Makes clarification welcome rather than an interruption to the queue.

Beside the main label

Identifies the physical source without interpreting it.

Has accepted

Gives the question a confirmed professional owner.

Phrases you can actually use

C Notice the language / 2 of 2

EXPLAIN THE HANDOFF

"The pharmacist will discuss what the instruction means for your medicine."

CLARIFY THE UNCERTAINTY

"Please tell Ravi which part is unclear."

AVOID A FALSE COMPLETION

"The question is accepted; the consultation has not yet been completed."

KEEP PICKUP AND COUNSELING DISTINCT

"Handing over a package does not itself answer a clinical question."

RESPECT PRIVACY

"We can use the practice's appropriate private discussion arrangement."

INVITE CHECKING UNDERSTANDING

"You can explain what you understood and ask Ravi to clarify it."

ROUTE DEVICE QUESTIONS

"Ravi can address the appropriate use of the supplied device within the consultation."

CLOSE WITH THE OWNER

"I will connect you with Ravi, who has accepted the question."

Not automatically optional

Corrects an inference without supplying a clinical instruction.

Explain what you understood

Supports clarification instead of testing the patient's intelligence.

Not yet completed

Keeps the handoff distinct from the actual consultation.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which response completes the appropriate handoff?

- A** Ravi has accepted your auxiliary-label question and is available at the consultation counter.
- B** Ignore the extra label because it is not the main one.
- C** I will invent an interpretation to keep the queue moving.
- D** Collecting the package means counseling is complete.

2. What does Ravi's acceptance establish?

- A** The patient already understands every instruction.
- B** A dose change has been prescribed.
- C** The label can be removed.
- D** The question has a receiving pharmacist, not a completed consultation.

3. Which statement is inappropriate?

- A** No instruction has been changed here.
- B** Device questions can be raised with the pharmacist.
- C** The auxiliary label is always optional.
- D** The exact question should reach Ravi.

4. What should the patient be encouraged to clarify?

- A** An invented dose supplied by the technician
- B** Another patient's instructions as a substitute
- C** The meaning of an unnamed medicine without a professional review
- D** The actual unclear wording and their understanding during the pharmacist discussion

Answers and explanations: [page 93](#).

The label question has a named pharmacist

E Conversations / turns 1-10

05

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

privacy / accepted handoff / patient information leaflet / teach-back / consultation counter / administration device / counseling completion / dispensing label / auxiliary label / patient counseling

- 01 **Arden:** Kit, before I finish collecting this, what does the extra label mean? I recognize the words, but I am not sure how they apply to me.
- 02 **Kit:** Thank you for asking. You are referring to the ¹ _____ beside the main label. I can identify it, but pharmacist Ravi needs to explain the instruction for your medicine.
- 03 **Arden:** I assumed that if it was important, everything would be on the main label. Is an extra sticker just general advice I can ignore?
- 04 **Kit:** It is not automatically optional. Ravi can explain how the ² _____ and additional information apply together. A label's position on the package is not a reason to disregard it.
- 05 **Arden:** Could you just tell me quickly? Everyone behind me is waiting, and I feel awkward asking for another conversation.
- 06 **Kit:** Your question deserves proper ³ _____. Ravi has accepted it and is available. I will connect you with him; I am not authorized to interpret the instruction.
- 07 **Arden:** Where should I go? I want to know the person is expecting me and understands the question.
- 08 **Kit:** Ravi is at the ⁴ _____. I will make clear that your question is about this auxiliary label, so the handoff includes the issue rather than only your arrival.
- 09 **Arden:** That helps. Does accepting the question mean somebody has already checked everything with me, or simply that Ravi is going to discuss it?
- 10 **Kit:** It is an ⁵ _____, not a completed discussion. We should not record counseling as finished just because the question was passed on or a package reached the pickup counter.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

05

- 11 **Arden:** The leaflet says it differently. Can I show Ravi both? I am not sure whether they mean the same thing.
- 12 **Kit:** Bring the ⁶ _____ into the consultation question. Ravi can address the actual wording and context; I should not resolve an apparent difference by choosing an instruction for you.
- 13 **Arden:** There is a device with the package too. Could I ask about it in the pharmacist conversation?
- 14 **Kit:** Yes. A question about the ⁷ _____ belongs in the appropriate consultation. I will not assume that recognizing the package means you have already been shown or understand the device.
- 15 **Arden:** I would prefer to ask the more personal part somewhere people in the queue cannot easily hear. Is that possible through the pharmacy's arrangements?
- 16 **Kit:** We can protect your ⁸ _____. I will tell Ravi that you would like a private discussion without announcing your question to everyone here.
- 17 **Arden:** Once Ravi explains it, I may need to repeat what I understood. Sometimes I say yes because I heard the words, but the meaning is still unclear.
- 18 **Kit:** That kind of ⁹ _____ can help Ravi clarify the explanation. You can say what you understood and ask about anything that does not fit, rather than feel obliged to pretend.
- 19 **Arden:** Thank you. Please keep the original directions unchanged and connect me with Ravi. I want the meaning clarified, not a new instruction guessed at this counter.
- 20 **Kit:** No direction has been changed. We will distinguish the handoff from ¹⁰ _____, and Ravi will address the question through the consultation. I will preserve the label reference and your request for a clear explanation.

ANSWERS / EXPLANATIONS: PAGE 94

1. auxiliary label | 2. dispensing label | 3. patient counseling | 4. consultation counter | 5. accepted handoff | 6. patient information leaflet | 7. administration device | 8. privacy | 9. teach-back | 10. counseling completion

Hand off a storage-label question

F Say it

Patient Drew asks about storage wording. The technician cannot interpret it. Pharmacist Noor accepts the question and is available in the consultation area. No instruction is changed.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

consultation / unchanged / Noor / storage

Technician: "The question concerns the ¹ _____ wording."

Patient: "The pharmacist who accepted it is ² _____."

Technician: "Please use the ³ _____ area."

Technician: "The directions remain ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 94

1. storage | 2. Noor | 3. consultation | 4. unchanged

Rehearse with a fixed script

1. Read the label question and named pharmacist handoff without adding a medicine instruction.
2. Complete the storage transfer with Noor and the consultation area.
3. Check all four answers, then repeat that the direction is unchanged and the consultation has not yet happened.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A neighbor at the counter

A Read the situation

Explain a private verification step without accusing the collector or disclosing a patient medication list.

Pavel asks to collect a prescription for a neighbor. Technician Vera has not yet verified the collection arrangement under the pharmacy process. Pavel asks Vera to read the neighbor's medicines aloud so he can recognize the correct package. Lead pharmacist Maya is available to review the request privately. No pickup decision or contact permission has been established. The task is not to announce a universal ban on friends collecting prescriptions. Vera must protect information, avoid an improvised identity test, and give the request a clear next step.

PEOPLE / Pavel: Neighbor requesting collection | Vera: Pharmacy technician

06

EXPLAIN THE PROCESS WITHOUT IMPLYING WRONGDOING

A verification question can sound like an accusation when a collector believes they are helping. Acknowledge that purpose, identify the unresolved arrangement, and name the person who can review it. Privacy is not a reason to shame someone publicly.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What is unresolved?

A Whether friends are universally prohibited from collecting

C Whether every medicine may be read aloud

B Whether Pavel has committed an offense

D The collection arrangement under the pharmacy process

2. Who can review the request privately?

A Lead pharmacist Maya

C Pavel acting alone

B An unidentified person in the queue

D A delivery driver outside

3. Which requested action should Vera avoid?

A Explaining that review is needed

C Reading the medication list aloud as an improvised identification test

B Connecting Pavel with Maya

D Acknowledging Pavel's reason for coming

Answers and explanations: page 95.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

collection arrangement

The agreed or permitted process for another person to collect an item.

IN USE verify the collection arrangement

designated collector

Person identified to collect on someone else's behalf.

IN USE confirm the designated collector

identity verification

Checking a person's identity using the applicable process.

IN USE complete identity verification

authority to collect

Basis on which a person may receive another person's prescription.

IN USE clarify authority to collect

professional judgment

Qualified assessment using relevant facts and applicable standards.

IN USE exercise professional judgment

reasonable inference

Conclusion supported by circumstances rather than certainty from a single claim.

IN USE make a reasonable inference

disclosure

Making information available to another person.

IN USE limit the disclosure

confidentiality

Duty to protect information from inappropriate access or sharing.

IN USE maintain confidentiality

patient preference

A person's expressed choice about care or communication.

IN USE record a patient preference

contact permission

Established basis for contacting or sharing through a particular person or channel.

IN USE verify contact permission

preferred channel

Communication method the patient has chosen where appropriate.

IN USE confirm the preferred channel

alternate contact

Another person or destination used when authorized and suitable.

IN USE verify an alternate contact

Precision check

Identity, relationship, and authority answer different questions. Knowing a person is a neighbor does not by itself settle every disclosure. Nor does uncertainty create a universal ban on third-party collection. Apply the actual pharmacy process and qualified judgment.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

voicemail

Recorded message left when a call is unanswered.

IN USE check voicemail preferences

callback number

Number to use for a return call after appropriate verification.

IN USE verify the callback number

relationship claim

Statement that someone has a particular connection to a patient.

IN USE assess the relationship claim

pickup request

Request to receive a prepared prescription from the pharmacy.

IN USE review a pickup request

private review

Assessment conducted with appropriate protection from public hearing.

IN USE arrange a private review

public counter

Customer-facing space where other people may hear a conversation.

IN USE avoid details at the public counter

medication list

Record of medicines associated with a patient.

IN USE protect the medication list

relevant information

Details needed for the specific permitted purpose.

IN USE share relevant information

identity mismatch

Difference between identifying details that requires clarification.

IN USE resolve an identity mismatch

authorization record

Documentation relevant to permission for a specified action.

IN USE check an authorization record

third party

Person or organization other than the main parties to a matter.

IN USE verify a third-party request

verified destination

Contact address or number checked through the appropriate process.

IN USE use a verified destination

Precision check

US health-privacy guidance allows professional judgment in appropriate friend or family prescription pickups; it does not impose advance written authorization in every case. This exercise concerns a still-unresolved request, not a new legal rule for every pharmacy or country.

Phrases you can actually use

C Notice the language / 1 of 2

ACKNOWLEDGE THE PURPOSE

"I understand that you have come to help your neighbor."

AVOID AN ACCUSATION

"I am checking the arrangement, not accusing you of anything."

OFFER THE NEXT STEP

"Maya can review the request privately."

SEPARATE IDENTITY AND AUTHORITY

"Confirming who you are does not answer every collection question."

NAME THE UNRESOLVED STEP

"We have not yet verified the collection arrangement."

PROTECT THE LIST

"I cannot use a public reading of the medication list as an identification test."

AVOID A BLANKET BAN

"I am not saying friends can never collect prescriptions."

KEEP THE PATIENT CENTRAL

"We need to handle the patient's information through the appropriate process."

Have not yet

States an unresolved step rather than a permanent refusal.

Not accusing

Repairs an interpersonal misunderstanding without abandoning verification.

As an identification test

Explains why the proposed disclosure is inappropriate.

Phrases you can actually use

C Notice the language / 2 of 2

CHECK THE CHANNEL

"We have not established permission to use that contact method."

LIMIT THE CONVERSATION

"We can discuss the request without announcing medicine names here."

REFER THE JUDGMENT

"Maya can assess the relevant circumstances."

MAKE THE HANDOFF CONCRETE

"I will connect you with Maya and explain the unresolved collection request."

DO NOT IMPROVISE A TEST

"Please let us follow the pharmacy's verification process."

CLARIFY THE DECISION STATUS

"A collection decision has not yet been made."

PROTECT CONTACT DETAILS

"I will not read out another person's contact information."

CLOSE COURTEOUSLY

"Thank you for waiting while we resolve this privately."

06

Can review

Offers a practical next step without promising approval.

Not saying friends can never

Avoids turning a local uncertainty into a universal legal claim.

Relevant circumstances

Leaves professional judgment tied to actual evidence.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

06

1. Pavel says, "So friends are never allowed to collect?" Which response is accurate?

- A** We can settle it by announcing all the medicines.
- B** That is not what I am saying; Maya can review this unresolved request privately.
- C** Correct, every friend is prohibited everywhere.
- D** Any claimed friendship automatically permits every disclosure.

2. Why does confirming Pavel's identity not finish this review?

- A** A familiar collector's identity removes the need to follow the pharmacy process.
- B** It establishes who he is, while this collection arrangement still needs review.
- C** A confirmed identity makes every medication-list disclosure permissible.
- D** Identity verification automatically authorizes every future collection.

3. What can Vera promise now?

- A** Disclosure of the medication list
- B** Permission to contact any relative
- C** Connection with Maya for private review
- D** Approval of the collection

4. Which note is factual?

- A** Neighbor collection requested; arrangement unresolved; referred to Maya privately.
- B** Unauthorized stranger confirmed to be stealing medicines.
- C** Written authorization is legally mandatory in every pickup.
- D** Patient consented to every contact method.

Answers and explanations: [page 95](#).

A neighbor at the counter

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

identity verification / confidentiality / pickup request / professional judgment / private review / medication list / authority to collect / contact permission / relevant information / collection arrangement

06

- 01 **Pavel:** I am collecting for my neighbor. She asked me to stop here on my way home, but I do not know which package I need.
- 02 **Vera:** I understand. We need to verify the ¹ _____ through our process. I can ask lead pharmacist Maya to review your request privately.
- 03 **Pavel:** Could you just read out her medicines? I will probably recognize one of the names, and that would save us both some time.
- 04 **Vera:** I cannot use the ² _____ as a public identification test. We can take the request forward without announcing those names to everyone at the counter.
- 05 **Pavel:** Do you think I am lying? She asked me to help, and now I feel as though I have done something wrong.
- 06 **Vera:** No. I am not accusing you. The ³ _____ needs review, and Maya is available to handle it with you. I appreciate why you have come.
- 07 **Pavel:** Does that mean friends and neighbors are never allowed to collect, or just that you have not settled the arrangement for this visit?
- 08 **Vera:** It means this request is unresolved. I am not announcing a blanket ban. Maya can use the applicable process and ⁴ _____ in reviewing it.
- 09 **Pavel:** I can tell you my name and explain the relationship. Is proving who I am the same thing as proving that I can collect?
- 10 **Vera:** Those are related but distinct. ⁵ _____ addresses who you are. The appropriate review also needs to resolve the collection arrangement without using medicine names as a guessing game.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

- 11 **Pavel:** Would calling her help? I have a number on my phone, but I do not know what contact details or preferences your pharmacy holds.
- 12 **Vera:** We should use the appropriate process for ⁶ _____ and verified details. I will not assume that every number or channel offered at the counter is suitable.
- 13 **Pavel:** Can we move away from the queue? I can explain why she could not come, but I would rather not announce it here.
- 14 **Vera:** Yes. Maya can provide a ⁷ _____. I will pass on the collection question without describing your neighbor's medicines or circumstances to the waiting customers.
- 15 **Pavel:** Please make sure she knows that I am here to collect, rather than telling her that I asked for permission to see every record.
- 16 **Vera:** I will keep the ⁸ _____ accurate: you requested collection for your neighbor, and we have not yet resolved the arrangement. That is the issue Maya will receive.
- 17 **Pavel:** Thank you. I can wait for that conversation. I mainly needed to know whether there was a next step instead of simply being turned away.
- 18 **Vera:** There is. We will protect ⁹ _____ while Maya reviews it. I cannot promise the outcome before that review, but I can connect you with her.
- 19 **Pavel:** Then please do that. I understand that you have not approved or refused the collection yet and that we should discuss the details privately.
- 20 **Vera:** Exactly. The ¹⁰ _____ will be addressed through the appropriate review. I will explain where the request stands and introduce you to Maya.

06

ANSWERS / EXPLANATIONS: PAGE 96

1. collection arrangement | 2. medication list | 3. pickup request | 4. professional judgment | 5. identity verification | 6. contact permission | 7. private review | 8. relevant information | 9. confidentiality | 10. authority to collect

Clarify an alternate contact request

F Say it

Collector Sam supplies a new telephone number. Contact permission is unverified. Pharmacist Ellis accepts the private review. No patient information has been disclosed.

Complete the four gaps. Use each bank entry once.

06

WORD + PHRASE BANK

unverified / disclosed / telephone number / Ellis

Technician: "The new ¹ _____ has not been verified."

Collector: "Contact permission is still ² _____."

Technician: "Pharmacist ³ _____ has accepted the review."

Technician: "No patient information has been ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 96

1. telephone number | 2. unverified | 3. Ellis | 4. disclosed

Rehearse with a fixed script

1. Read the private-collection exchange, keeping the courteous acknowledgment and unresolved arrangement together.
2. Complete the alternate-number transfer without assuming that a supplied number establishes contact permission.
3. Check the key, then repeat the private handoff without reading patient details to the queue.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

AB41 is not AB14

A Read the situation

Read back product identifiers and distinguish an unverified stock match from a confirmed recall assessment.

A fictional recall notice names lot AB41. The delivery record shows AB14, but nobody has checked the label on the stock itself. Technician Emil brings the discrepancy to pharmacist Maya, who owns the recall assessment. The team must compare the actual product and lot with the complete notice through its approved process. Neither affected-stock status nor patient exposure has been established. A different number in one record does not prove the shelf is clear. No disposal, release, patient-contact, or treatment decision is supplied.

PEOPLE / Emil: Pharmacy technician | Maya: Lead pharmacist

07

READ IDENTIFIERS, NOT IMPRESSIONS

A familiar-looking code can be misheard when colleagues are busy. Read the exact sequence, name the source, and ask for a read-back. Correcting AB41 to AB14 is a factual distinction, not a criticism of the other person.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which source has not been checked?

- A The notice naming AB41
- B The identity of the assessment owner
- C The label on the actual stock
- D The delivery record showing AB14

2. What can be concluded now?

- A A discrepancy needs assessment; affected-stock status is not established.
- B All stock is unaffected.
- C Every patient received recalled medicine.
- D The notice must be wrong.

3. Who owns the recall assessment?

- A The patient queue
- B An unidentified supplier driver
- C Pharmacist Maya
- D Emil acting beyond the agreed process

Answers and explanations: [page 97](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

recall notice

Formal communication identifying a product problem and applicable actions.

IN USE review the recall notice

lot number

Identifier for a defined production group.

IN USE read back the lot number

batch

Defined quantity produced under a specified manufacturing process.

IN USE trace the batch

serial number

Identifier assigned to a particular individual item or package.

IN USE verify a serial number

product identifier

Code or set of details identifying a product.

IN USE match the product identifier

National Drug Code

US identifier commonly abbreviated NDC for a listed drug product.

IN USE compare the National Drug Code

manufacturer

Organization responsible for making a product.

IN USE confirm the manufacturer

expiration date

Date after which a product should not be used under its labeling.

IN USE check the expiration date

delivery record

Documentation of goods delivered to a location.

IN USE reconcile the delivery record

physical stock

Actual inventory present rather than entries in a system.

IN USE inspect the physical stock

stock reconciliation

Comparison of records with actual inventory to resolve differences.

IN USE complete stock reconciliation

traceability

Ability to follow product history and movement through records.

IN USE preserve traceability

Precision check

Lot, serial, and product identifiers are not interchangeable. A product code identifies the product category or configuration; a lot identifies a production group. The complete recall notice determines which details matter. A matching fragment is not the full assessment.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

quarantine

Controlled segregation or status preventing use pending an authorized decision.

IN USE apply the quarantine process

affected stock

Inventory confirmed to fall within the relevant notice or problem scope.

IN USE identify affected stock

recall scope

Defined products, lots, dates, or other limits of a recall.

IN USE establish the recall scope

disposition

Authorized decision about what happens to a product.

IN USE record the disposition

return authorization

Permission or reference required for an approved product return.

IN USE obtain return authorization

destruction record

Documentation that product was destroyed through the authorized process.

IN USE retain a destruction record

release decision

Authorized decision allowing stock to become available for its intended use.

IN USE document a release decision

patient exposure

Situation in which a patient received or used a relevant product.

IN USE assess patient exposure

first-expiry-first-out

Using the earliest-expiring eligible stock first; FEFO does not permit expired or quarantined stock.

IN USE apply first-expiry-first-out

temperature excursion

Temperature outside specified storage limits.

IN USE report a temperature excursion

tamper evidence

Visible feature intended to show that packaging may have been interfered with.

IN USE check tamper evidence

quality defect

Product problem affecting conformity with relevant quality requirements.

IN USE report a quality defect

Precision check

Quarantine, return, destruction, and release are different controlled actions. Use the actual notice and pharmacy procedures, not this language exercise as an operational instruction. Questions about a patient taking medicine belong with the appropriate professional; a recall is not universal advice to stop treatment.

Phrases you can actually use

C Notice the language / 1 of 2

NAME THE DOCUMENT

"The notice names lot AB41."

READ BACK PRECISELY

"That is A, B, four, one on the notice."

AVOID PREMATURE CLEARANCE

"The record difference does not establish that the shelf is unaffected."

COMPARE THE FULL SCOPE

"We need the complete product details and notice scope."

READ THE SECOND SOURCE

"The delivery record shows AB14."

STATE THE MISSING CHECK

"The actual stock label has not yet been checked."

NAME THE OWNER

"Maya owns the recall assessment."

SEPARATE RECORDS AND STOCK

"A delivery entry is not a physical-stock check."

07

Shows versus confirms

A record shows an entry; confirmation requires the appropriate verification.

Not yet checked

Names the missing work without implying the result.

Four, one

Reads the actual sequence rather than a vague resemblance.

Phrases you can actually use

C Notice the language / 2 of 2

KEEP EXPOSURE UNCONFIRMED

"Patient exposure has not been established."

AVOID AN INVENTED DISPOSITION

"No disposal or release decision has been supplied."

REPORT A MISMATCH

"These identifiers differ; the reason is not yet known."

ROUTE CLINICAL QUESTIONS

"A pharmacist must address questions about taking the medicine."

PROTECT TRACEABILITY

"Preserve the original references and the results of each check."

FOLLOW THE REAL PROCEDURE

"Use the actual notice and the approved pharmacy process."

KEEP ACTIONS DISTINCT

"Holding stock is not the same as destroying it."

CLOSE THE RECORD ACCURATELY

"Record the findings and authorized decision when they are established."

Has not been established

Prevents uncertainty from becoming either reassurance or an alarm.

Complete product details

Expands the comparison beyond one code fragment.

When established

Keeps later decisions out of the current factual record.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which read-back preserves the notice code?

- A** A, B, one, four
- B** A, B, fourteen on both documents
- C** A, B, forty-one on the delivery record
- D** A, B, four, one

2. Which statement overstates the evidence?

- A** The shelf is clear because the delivery record shows a different lot.
- B** The actual stock label has not been checked.
- C** Maya owns the assessment.
- D** Full notice scope must be compared.

3. What should be preserved?

- A** An assumed exposure conclusion
- B** An invented disposal instruction
- C** The source identifiers, actual findings, and authorized decisions
- D** Only the newest number with the earlier one deleted

4. Which patient-facing claim is unsupported?

- A** The actual notice determines the relevant scope.
- B** Patient exposure is not established here.
- C** Every recall means you should stop taking medicine immediately.
- D** A pharmacist should address treatment questions.

Answers and explanations: [page 97](#).

AB41 is not AB14

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

recall notice / recall scope / Patient exposure / release decision / delivery record / lot number / traceability / Quarantine / physical stock / disposition

07

- 01 **Emil:** Maya, I have a code discrepancy on the recall paperwork. The notice and delivery record look similar, but the final two characters are reversed.
- 02 **Maya:** Start with the ¹ _____. Read its lot exactly, then name the second source. We should not let a familiar-looking code substitute for an exact comparison.
- 03 **Emil:** The notice says A, B, four, one: AB41. The delivery record says A, B, one, four: AB14. I have not checked the shelf label.
- 04 **Maya:** The ² _____ on the notice is AB41; the record shows AB14. Thank you for separating them. The physical label is still an outstanding check.
- 05 **Emil:** I was about to mark unaffected because AB14 is different. Then I realized I had only checked the delivery entry, not the cartons themselves.
- 06 **Maya:** It would jump ahead. The ³ _____ is one source. It does not establish which lot is on the shelf or complete the assessment against the notice.
- 07 **Emil:** Then I will not tell the counter team that the shelf is clear. What should I include when I describe the unresolved query?
- 08 **Maya:** Name both sources and the unchecked ⁴ _____. Also identify me as the assessment owner so colleagues know where findings and questions must go.
- 09 **Emil:** The product has other identifying details as well. I assume the lot code alone is not a reason to skip the product and manufacturer comparison.
- 10 **Maya:** Correct. We need the complete ⁵ _____ and applicable product details. Follow the actual notice and our approved process rather than infer the scope from two letters and two digits.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 64.

- 11 **Emil:** If we find a match, should I immediately describe it as proof that a patient received the affected product, or is that another question?
- 12 **Maya:** That is another question. ⁶ _____ has not been established. Stock findings and patient-related assessment must be recorded accurately without turning one into an unsupported conclusion about the other.
- 13 **Emil:** I will attach both source references. Otherwise the next shift may see a corrected summary without understanding why we raised the query.
- 14 **Maya:** Exactly. Preserve ⁷ _____ through the source references and actual findings. Anyone reviewing the matter needs to see what was checked, what differed, and what remains unresolved.
- 15 **Emil:** I also want to avoid using hold, return, and destroy as if they were the same action. The required action depends on the actual instructions.
- 16 **Maya:** Yes. ⁸ _____ and final disposition are distinct. Use the applicable process and notice; do not invent disposal or release instructions from this discrepancy.
- 17 **Emil:** If a patient asks whether to stop taking a medicine while we check this, I should connect that question with the appropriate pharmacist advice.
- 18 **Maya:** Correct. Do not turn an inventory query into a treatment instruction. An authorized ⁹ _____ for stock and clinical advice for a patient are different matters.
- 19 **Emil:** My summary will say AB41 on the notice, AB14 in the delivery record, stock label unchecked, and assessment with Maya. No affected-stock conclusion yet.
- 20 **Maya:** That is accurate. The ¹⁰ _____ is not established either. Bring the verified findings through our process, and keep the pending status clear until the responsible decision is recorded.

07

ANSWERS / EXPLANATIONS: PAGE 98

1. recall notice | 2. lot number | 3. delivery record | 4. physical stock | 5. recall scope | 6. Patient exposure | 7. traceability | 8. Quarantine | 9. disposition | 10. release decision

Read back a different lot discrepancy

F Say it

Notice lot CD27 differs from delivery entry CD72. Actual stock is unchecked. Pharmacist Noor owns the assessment. No affected-stock conclusion has been reached.

Complete the four gaps. Use each bank entry once.

07

WORD + PHRASE BANK

CD27 / unchecked / Noor / CD72

Technician: "The notice names ¹ _____."

Pharmacist: "The delivery entry is ² _____."

Technician: "The actual stock is still ³ _____."

Pharmacist: "The assessment owner is ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 98

1. CD27 | 2. CD72 | 3. unchecked | 4. Noor

Rehearse with a fixed script

1. Read AB41 and AB14 character by character, attaching each to the correct source.
2. Repeat the transfer with CD27 and CD72, the unchecked stock, and Noor.
3. Check every blank, then repeat the difference between a document mismatch and a confirmed affected-stock finding.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Correcting a ready message

A Read the situation

Correct an inaccurate status promptly, acknowledge its effect, and hand over a named follow-up without promising collection.

At 14:00, technician Jo told patient Asha that a prescription was ready after misreading a queue entry. At 14:10, Jo confirms that pharmacist review is still pending. Asha is already traveling to the pharmacy. Jo owns the correction call now and a status update at 16:00. No collection time is confirmed. Jo must clearly withdraw the earlier ready statement, explain the verified status, acknowledge the disruption, and preserve an accurate record. The team must distinguish an attempted call, successful contact, and a completed correction.

PEOPLE / Jo: Pharmacy technician | Asha: Patient

08

APOLOGIZE WITH THE CORRECTED FACT

A vague apology can leave the original message intact. State what was wrong and what is now verified before describing follow-up. Taking responsibility for a misread queue entry is more useful than blaming the system or defending the original wording.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What was wrong with the 14:00 message?

- A** It said ready although pharmacist review was still pending.
- B** It correctly guaranteed collection at 16:00.
- C** It confirmed a patient had already collected.
- D** It documented a completed correction call.

2. Which time is an update commitment?

- A** A guaranteed review completion time
- B** 16:00
- C** 14:00 as a new pickup slot
- D** Any assumed arrival time

3. What must a handoff distinguish?

- A** A queue entry and automatic readiness
- B** An apology and a clinical decision
- C** Travel already started and guaranteed supply
- D** Attempted contact and a correction actually communicated

Answers and explanations: page 99.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

queue entry

Item displayed in a workflow waiting for a processing step.

IN USE check the queue entry

workflow stage

Defined point within a sequence of work.

IN USE verify the workflow stage

status correction

Replacement of an inaccurate status with the verified one.

IN USE issue a status correction

ready notification

Message stating that an item is ready for collection.

IN USE withdraw a ready notification

pharmacist review

Professional review by the pharmacist within the dispensing process.

IN USE await pharmacist review

final check

Required final verification under the applicable dispensing process.

IN USE confirm the final check

in progress

Started but not completed.

IN USE mark the task in progress

pending

Awaiting an event, action, or decision.

IN USE show the review as pending

dispensed

Prepared or supplied through the relevant dispensing process; usage must be clear.

IN USE distinguish dispensed from collected

collected

Received by the person collecting the item.

IN USE record the item as collected

timestamp

Recorded time associated with an event or entry.

IN USE preserve the timestamp

contact attempt

Effort to reach someone without assuming contact succeeded.

IN USE record a contact attempt

Precision check

Ready, in progress, dispensed, and collected may appear close together in a system but describe different events. Use the pharmacy definitions and verified workflow state. Seeing an entry in a queue does not prove that review or release is complete.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

successful contact

Confirmed communication with the intended person through the appropriate process.

IN USE document successful contact

message delivered

Information actually conveyed, not merely prepared or queued.

IN USE confirm the message was delivered

acknowledgment

Response showing receipt or recognition of information.

IN USE obtain acknowledgment

correction owner

Person responsible for communicating and recording a correction.

IN USE name the correction owner

update commitment

Promise to provide information at a stated time or trigger.

IN USE keep the update commitment

readiness estimate

Forecast of when an item may be ready, not confirmation.

IN USE qualify a readiness estimate

handoff record

Written account transferring relevant facts and responsibility.

IN USE complete the handoff record

open action

Task that remains unfinished.

IN USE assign the open action

closed-loop handoff

Transfer in which receipt and responsibility are confirmed.

IN USE use a closed-loop handoff

service recovery

Actions addressing a service failure and its effects.

IN USE support service recovery

audit trail

Preserved sequence showing events and changes to a record.

IN USE maintain the audit trail

verified status

State established through the appropriate check.

IN USE communicate the verified status

Precision check

A correction is not complete merely because someone drafted a message or attempted a call. Record the actual contact result and remaining action. Preserve the earlier event and correction in the appropriate record instead of silently rewriting history.

Phrases you can actually use

C Notice the language / 1 of 2

OPEN THE CORRECTION

"I need to correct the ready message I gave you at 14:00."

TAKE RESPONSIBILITY

"I misread the queue entry and gave you an inaccurate status."

WITHDRAW THE PROMISE

"Please do not rely on my earlier statement that it was ready."

DEFINE THE UPDATE

"I will provide a status update at 16:00."

STATE THE VERIFIED POSITION

"Pharmacist review is still pending."

APOLOGIZE PLAINLY

"I am sorry that my message caused this disruption."

AVOID A REPLACEMENT GUESS

"I cannot confirm a collection time now."

KEEP THE DISTINCTION

"That is an update time, not a pickup appointment."

08

I need to correct

Signals that the earlier information must be replaced.

I misread

Takes responsibility without inventing a system fault.

Do not rely on

Explicitly withdraws the earlier claim rather than merely adding a vague caution.

Phrases you can actually use

C Notice the language / 2 of 2

CHECK RECEIPT

"Can I confirm that you received the corrected status?"

RECORD THE ACTUAL OUTCOME

"Record whether contact succeeded and what was communicated."

MAKE THE HANDOFF EXPLICIT

"Any transfer must include the open action and an accepting owner."

DO NOT CLAIM CLINICAL COMPLETION

"I cannot speak as though pharmacist review has finished."

RESPECT SAFE CONTACT

"Please respond only when it is safe and appropriate."

NAME THE OWNER

"I own the correction and the 16:00 follow-up."

PRESERVE THE EARLIER EVENT

"Keep the original message and the correction visible in the record."

CLOSE WITH THE NEXT FACT

"The current status is pending review, with an update at 16:00."

08

Update time, not pickup

Prevents a new expectation from replacing the old mistake.

Whether contact succeeded

Separates effort from result in the record.

Accepting owner

Makes a shift handoff a confirmed transfer rather than a name in a note.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which opening most clearly repairs the error?

- A** There might have been some confusion, but come anyway.
- B** Everything is fine because I sent a message.
- C** Ready probably means ready enough.
- D** I need to correct my 14:00 ready message: pharmacist review is still pending.

2. Which promise is supported?

- A** Jo will provide a status update at 16:00.
- B** The prescription will be ready at 16:00.
- C** Review is guaranteed to finish before Asha arrives.
- D** Asha has already received the medicine.

3. If the call is unanswered, what should the record show?

- A** A completed correction communicated to Asha
- B** Asha's agreement to the revised status
- C** A confirmed collection time
- D** An attempted contact with the correction action still unresolved

4. Which handoff is complete in principle?

- A** A new name with no acknowledged responsibility
- B** An erased original message
- C** Only the word sorry
- D** Verified status, actual contact outcome, open follow-up, and accepting owner

Answers and explanations: [page 99](#).

Correcting a ready message

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

update commitment / pharmacist review / verified status / closed-loop handoff / readiness estimate / successful contact / status correction / audit trail / open action / ready notification

08

- 01 **Jo:** Asha, this is Jo at the pharmacy. Is it safe and appropriate to speak now? I need to correct the ready message I gave you earlier.
- 02 **Asha:** I can speak. I am already traveling because your ¹ _____ said I could collect. What has changed?
- 03 **Jo:** At 14:10 I checked the status and confirmed that pharmacist review is still pending. I misread the queue entry when I spoke to you at 14:00.
- 04 **Asha:** Then the ² _____ is that it is not confirmed ready. I need you to say that clearly because I arranged my afternoon around collecting it.
- 05 **Jo:** Yes. My earlier ready message was wrong. I am sorry you arranged travel because of it. Review is still pending, and I cannot give you a collection time.
- 06 **Asha:** Can you explain the difference between seeing it in the system and knowing it has completed ³ _____? That seems to be where the message went wrong.
- 07 **Jo:** I treated a queue entry as though it established readiness. It did not. I should have checked the actual stage before giving you a collection message.
- 08 **Asha:** I appreciate you owning the mistake. What is the ⁴ _____ now, and does that tell me when I can collect?
- 09 **Jo:** I own the correction call now and a status update at 16:00. That is a time for information, not a confirmed pickup appointment or promise that review will be complete.
- 10 **Asha:** Then please do not describe 16:00 as a ⁵ _____ if you do not have evidence for one. I would rather have an honest status than another trip based on a guess.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

- 11 **Jo:** Agreed. I cannot supply a readiness time. At 16:00 I will report the verified position, including anything still pending, rather than turn the update into another collection promise.
- 12 **Asha:** Will the record show that you actually reached me? I would not want a later colleague to confuse an attempted call with ⁶ _____ and assume I had been told.
- 13 **Jo:** Yes. I will record the actual contact result and the corrected information communicated through our process. An unanswered call would not mean that the correction had reached you.
- 14 **Asha:** If someone else takes over later, I want the ⁷ _____ to stay visible. Otherwise I may need to explain the entire problem again.
- 15 **Jo:** I remain responsible for the follow-up. If a transfer becomes necessary, it must include the verified status, what I communicated, the outstanding action, and a colleague who accepts responsibility.
- 16 **Asha:** That sounds like a ⁸ _____, rather than leaving a name in the system and hoping they notice.
- 17 **Jo:** Exactly. I will keep the original message and this correction in the record. The next shift needs to know what you were told, not just the latest status.
- 18 **Asha:** Keep the ⁹ _____. I have received the correction: review is pending, there is no collection time, and you will update me at 16:00.
- 19 **Jo:** That is correct. Thank you for confirming receipt. I am sorry for the disruption. I will keep the follow-up assigned and communicate the verified status at the agreed time.
- 20 **Asha:** All right. Please make the ¹⁰ _____ clear to anyone handling my enquiry, so the original ready message is not repeated when I next contact the pharmacy.

08

ANSWERS / EXPLANATIONS: PAGE 100

1. ready notification | 2. verified status | 3. pharmacist review | 4. update commitment | 5. readiness estimate | 6. successful contact | 7. open action | 8. closed-loop handoff | 9. audit trail | 10. status correction

Correct a queued message

F Say it

At 10:05, technician Erin discovers an inaccurate ready message prepared for patient Leo. It has not been sent. Review is pending. Erin owns correcting the queued message before release.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

release / sent / Erin / pending

08

Erin: "The inaccurate message has not been ¹ _____."

Colleague: "The verified review status is ² _____."

Erin: "The correction owner is ³ _____."

Colleague: "Correct it before message ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 100

1. sent | 2. pending | 3. Erin | 4. release

Rehearse with a fixed script

1. Read the correction call with the original error, pending review, and 16:00 update clearly audible.
2. Complete the transfer in which the message has not been sent; do not invent a patient contact that never occurred.
3. Check the key, then repeat the distinction between correcting a queued message and communicating a correction to a patient.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Check what the number measures

E More workplace conversations / 1-10

Explain a days-supply calculation using verified units and distinguish a maximum-use calculation from actual consumption.

Fictional calculation check, not patient advice: order A supplies 60 tablets, strength 5 mg each, verified direction one tablet twice daily. A copied days-supply field says 60. Separate order B supplies 30 tablets, as needed, with an explicit maximum of 6 tablets daily. Pharmacist review is required before processing.

PEOPLE / Tara: Pharmacy technician | Omar: Pharmacist

Use each bank entry once, without changing its form. Answers follow turn 20.

09

WORD + PHRASE BANK

Adherence / clinical approval / strength / days supply / directions for use / maximum daily use

- 01 **Tara:** Omar, order A has sixty tablets, but the days-supply field also says sixty. That looks copied from the old once-daily order. The current direction is twice daily.
- 02 **Omar:** Start with the current ¹ _____. Read the amount per administration and the frequency separately before you divide anything.
- 03 **Tara:** One tablet each time, twice daily: two tablets per day. Quantity sixty divided by two gives thirty days at that stated schedule.
- 04 **Omar:** Correct: the calculated ² _____ is thirty. What does the five-milligram figure describe, and why is it not your divisor here?
- 05 **Tara:** That is the strength per tablet. Dividing sixty tablets by five milligrams would mix unlike units and would not produce the number of days.
- 06 **Omar:** Exactly. Keep ³ _____, tablet quantity, and daily tablet use in separate fields. The arithmetic should carry the units, not just a row of numbers.
- 07 **Tara:** I will correct the draft calculation through our process and retain the reason: current twice-daily directions, not the earlier once-daily entry.
- 08 **Omar:** Bring it back for the required check. Correct arithmetic is not ⁴ _____ or evidence that all other dispensing checks are complete.
- 09 **Tara:** Order B is different. It says as needed, maximum six tablets in a day, quantity thirty. Should I describe that as certain to last five days?
- 10 **Omar:** No. Five days is the calculation at ⁵ _____. As-needed use does not tell us the amount the person will actually take each day.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 76.

- 11 **Tara:** So thirty divided by six gives five at the maximum, but lower use would leave some tablets after five days. I cannot infer the actual use pattern from that division.
- 12 **Omar:** Right. Nor should you tell the patient to take six daily because that makes the calculation easy. A maximum is a limit in the supplied instruction, not a new scheduled dose.
- 13 **Tara:** For the claim entry, I still need to apply the relevant plan and pharmacy rules, with your check. The language exercise does not establish a universal billing rule.
- 14 **Omar:** Yes. Keep the arithmetic and its assumptions visible. If the directions or maximum were unclear, we would resolve that rather than choose a convenient number to pass the claim.
- 15 **Tara:** Does a thirty-day calculation on order A establish that the patient took every dose in an earlier supply?
- 16 **Omar:** No. ⁶ _____ needs an appropriate discussion and assessment. A calculated supply period is not an observation of what somebody swallowed.
- 17 **Tara:** And if a patient reports taking it differently, I should record their account and bring it to you, not alter the directions just to make the stock balance.
- 18 **Omar:** Correct. Keep prescribed use, reported use, and the supplied quantity distinguishable. A discrepancy may need clarification; hiding it inside a new days-supply figure would lose important information.
- 19 **Tara:** My read-back: order A, sixty tablets at two daily, thirty days. Order B, thirty tablets at six maximum daily, five days at maximum use, actual duration unknown.
- 20 **Omar:** Agreed. Mark both calculations for the required review. No supply authorization, treatment change, or confirmed pattern of patient use follows solely from these numbers.

09

ANSWERS / EXPLANATIONS: PAGE 101

1. directions for use | 2. days supply | 3. strength | 4. clinical approval | 5. maximum daily use | 6. Adherence

Use tablets per day, not milligrams

F Say it in a new situation

Fictional arithmetic only: 84 tablets, strength 10 mg per tablet, verified direction one tablet three times daily. Separately, 24 tablets have as-needed directions with an explicit maximum of 4 daily. No actual consumption or billing outcome is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

unknown / 6 / 3 / 28

09

Technician: The scheduled order uses ¹ _____ tablets daily.

Pharmacist: Eighty-four tablets at that rate represents ² _____ days.

Technician: The separate as-needed quantity represents ³ _____ days at maximum use.

Pharmacist: Actual as-needed consumption remains ⁴ _____ .

ANSWERS / EXPLANATIONS: PAGE 101

1. 3 | 2. 28 | 3. 6 | 4. unknown

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

Read the whole temperature log

E More workplace conversations / 1-10

Report the recorded excursion interval, measurement source, stock details, and unresolved disposition without inferring product stability.

Fictional Product R requires 2 to 8 degrees Celsius. The validated fridge air log establishes 10:05 to 10:40 above 8, peak 11.4. Current reading at 11:00 is 5. Three cartons contain 12 vials each, lot Q28. Technician Lina has activated the hold and alerts pharmacist Daniel; no release decision exists.

PEOPLE / Lina: Pharmacy technician | Daniel: Pharmacist

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

thirty-five minutes / Quarantine / traceability / temperature excursion / air temperature / disposition

10

- 01 **Lina:** Daniel, Product R is on hold. The fridge is showing five degrees now, but its validated log records an earlier out-of-range interval. I have not released the stock.
- 02 **Daniel:** Give me the ¹ _____ details, including the measurement source. Do not describe the medicine itself as reaching a temperature unless we have that evidence.
- 03 **Lina:** The sensor measures fridge air. The established interval above eight is ten oh five to ten forty, with a peak of eleven point four degrees Celsius.
- 04 **Daniel:** That is ² _____ outside the specified upper limit. The present reading at eleven does not tell us when that interval began or how high it went.
- 05 **Lina:** I have preserved the data export, sensor identity, and timestamps. I will not reset the record before the evidence is retained under our procedure.
- 06 **Daniel:** Good. The ³ _____ tells us about the storage environment. The responsible assessment needs the product details as well as the log, not a guess about a vial's internal temperature.
- 07 **Lina:** Three cartons, twelve vials each, thirty-six vials in total. Product R, lot Q28. I am checking expiry and whether any earlier excursion is recorded.
- 08 **Daniel:** Retain that ⁴ _____. Unknown earlier exposure should stay unknown until checked; do not mark none merely because you have not found the record yet.
- 09 **Lina:** The hold is marked in the system and on the segregated stock. Should it stay under the product's required storage arrangements while the decision is pending?
- 10 **Daniel:** Follow our established hold and storage procedure. ⁵ _____ controls availability; it does not mean leaving temperature-sensitive stock unattended on a counter.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 79.

- 11 **Lina:** A colleague said another brand tolerated a warmer delivery last month. Can that earlier decision clear this lot too?
- 12 **Daniel:** No. I need information for the actual product, conditions, and history. Another brand's previous assessment is not a release decision for Product R after this event.
- 13 **Lina:** I will gather the current manufacturer information and record the source. I will also check whether any stock movement occurred during the interval, rather than assume everything stayed untouched.
- 14 **Daniel:** Bring the findings to me for the assessment. Do not tell patients to change treatment on the basis of this inventory conversation; any affected supply or clinical concern needs the appropriate separate response.
- 15 **Lina:** For the incident note, I will describe the reading and times without stating that the product is spoiled. The excursion is established, but suitability has not been determined.
- 16 **Daniel:** Correct. The ⁶ _____ remains pending. We need the evidence and the authorized decision, not a premature release or destruction entry.
- 17 **Lina:** The current five-degree reading also does not prove the equipment fault has been investigated. I will keep the fridge-status issue separate from the stock assessment.
- 18 **Daniel:** Exactly. Check the equipment through the relevant procedure and retain its findings. A repaired or stable fridge and an assessed medicine lot answer different questions.
- 19 **Lina:** Summary: thirty-six vials Q28 held; fridge-air peak eleven point four, thirty-five-minute interval; current five; earlier exposure being checked; product and equipment reviews pending.
- 20 **Daniel:** That is the report I need. I own the product assessment. Keep the hold and storage controls in place under our procedure until the authorized outcome is documented.

10

ANSWERS / EXPLANATIONS: PAGE 102

1. temperature excursion | 2. thirty-five minutes | 3. air temperature | 4. traceability | 5. Quarantine | 6. disposition

Do not substitute the current reading

F Say it in a new situation

Fictional required range: 2 to 8 degrees Celsius. An established logger interval above 8 runs 14:20 to 14:50; peak 10.2. Current reading at 15:00 is 4.8. Two cartons contain 15 vials each. Stock is held; no disposition is confirmed.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

30 / vials / 10.2 / pending

10

Technician: The established excursion lasted ¹ _____ minutes.

Pharmacist: The recorded peak was ² _____ degrees Celsius.

Technician: Two cartons of fifteen means thirty ³ _____ on hold.

Pharmacist: The authorized disposition is still ⁴ _____ .

ANSWERS / EXPLANATIONS: PAGE 102

1. 30 | 2. 10.2 | 3. vials | 4. pending

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.

2. Switch roles. Read it again with the word bank covered.

3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

A change before the next pack

E More workplace conversations / 1-10

Communicate a verified regimen change across current and future medicine packs without treating a printing update as completed patient communication.

Fictional case on Friday 9 October: pharmacist Ren verifies an authorized order discontinuing Medicine P from this evening. The patient's current multi-compartment pack covers 5 to 11 October; a prepared, undelivered pack covers 12 to 18 October. Both contain P. Ren owns immediate patient coordination; technician Jade controls the pending pack.

PEOPLE / Jade: Pharmacy technician | Ren: Pharmacist

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

contact outcome / pack identifier / multi-compartment pack / prepared / Medicines reconciliation / effective date

11

- 01 **Jade:** Ren, the new instruction discontinues Medicine P from this evening, but next week's pack is already prepared. It has not gone out. Does your verification cover the effective date too?
- 02 **Ren:** Yes: Friday the ninth, this evening. Put the prepared ¹ _____ on hold under our process. We must address the pack already at home as well.
- 03 **Jade:** The current pack runs from October fifth through the eleventh. It also contains P. Changing only the pack starting Monday would miss this evening and the weekend.
- 04 **Ren:** Exactly. The ² _____ and the next packaging cycle are different. I will coordinate the immediate patient instructions and support, not defer the issue to Monday's delivery.
- 05 **Jade:** I have applied the hold to the October twelfth pack and removed it from the dispatch queue. I have not opened, altered, or released it.
- 06 **Ren:** Keep the ³ _____ and covered dates in the record. A note saying pack changed would be too vague when there are two different packs to account for.
- 07 **Jade:** Should I phone and tell the patient to remove whichever tablet looks like P? There may be several similar-looking tablets in a compartment.
- 08 **Ren:** No. Do not improvise identification or removal instructions from appearance. I will arrange the appropriate patient-specific response with the relevant people under our process.
- 09 **Jade:** The amended order is verified, but patient contact has not happened yet. I will record those as separate stages instead of marking the whole change complete.
- 10 **Ren:** Correct. ⁴ _____ needs the actual current information, not an assumption that every record and pack updated automatically.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 82.

- 11 **Jade:** The care team also has a medicines administration record. I will flag the need to coordinate that record through the proper route, not edit their administration history myself.
- 12 **Ren:** Yes. Preserve the source, verified instruction, timing, and contacts. A new regimen should not erase the record of support or administration that actually occurred before the change.
- 13 **Jade:** For our preparation record, I will retain the earlier version and the dated correction. The revised pack still needs the applicable preparation and final checks.
- 14 **Ren:** And we must distinguish ⁵ _____, checked, released, and delivered. A revised printout alone is not a corrected pack in the patient's hands.
- 15 **Jade:** If the patient has other medicines outside the pack, the same change should not become a message to stop everything. The order concerns P only.
- 16 **Ren:** Right. I will explain the specific authorized change and check understanding in the actual conversation. You should not invent instructions for other medicines or assume that every product belongs inside the pack.
- 17 **Jade:** For the handoff, I have two open actions: your immediate coordination of the current pack and our held future pack's correction and checking.
- 18 **Ren:** I accept the current-pack action now. Record the actual ⁶ _____ when it occurs, including anything still unresolved. If contact fails, we must continue the appropriate escalation, not silently wait for delivery day.
- 19 **Jade:** Then the status is: current-pack response with you now; future pack held and undelivered; authorized stop effective this evening; no completed patient communication yet.
- 20 **Ren:** Confirmed. I am starting that coordination immediately. Keep the future pack blocked from dispatch until its required correction and checks are completed and the release is authorized.

11

ANSWERS / EXPLANATIONS: PAGE 103

1. multi-compartment pack | 2. effective date | 3. pack identifier | 4. Medicines reconciliation | 5. prepared | 6. contact outcome

A delivery date is not an effective date

F Say it in a new situation

An authorized change is verified to take effect Wednesday 14 October. The patient has a current pack for 12 to 18 October; the next pack starts 19 October and is undelivered. Pharmacist Leena owns immediate patient coordination. No contact outcome is recorded yet.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

18 / completed / 19 / 14

Technician: The change takes effect on ¹ _____ October.

Pharmacist: The current pack ends on ² _____ October.

Technician: The next pack begins on ³ _____ October.

Pharmacist: Patient communication is not yet confirmed as ⁴ _____ .

11

ANSWERS / EXPLANATIONS: PAGE 103

1. 14 | 2. 18 | 3. 19 | 4. completed

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

01 Briefing and language checks

Prescription intake without assumptions

A Briefing checks / page 4

1. Which field is unclear?

B. Strength

The current order has an unclear strength field, which requires appropriate clarification.

2. What does Ellis remember?

C. A blue label and a usual prescription pattern

The remembered label and pattern are reported context, not verified current instructions.

3. Who can take the clarification?

B. Pharmacist Maya

Maya is the identified pharmacist route, while Nia cannot choose the strength.

D Language checks / page 9

1. Which reply is accurate?

D. The strength is unclear; Maya can clarify it, and no completion time is confirmed.

The reply identifies the unresolved detail, appropriate route, and genuine timing uncertainty.

2. Which distinction is correct?

A. Strength describes product content per unit or quantity; dose describes the prescribed amount used.

These terms describe different information and cannot be substituted without creating ambiguity.

3. What should happen to the patient's recollection?

A. Record it as reported context for the pharmacist, not verified current instructions.

The recollection may help clarification while remaining distinct from an authorized current instruction.

4. What must Nia avoid promising?

D. A completion time that has not been confirmed

The brief provides no known completion time, so a definite timing promise would be unsupported.

01 Conversation answers

Prescription intake without assumptions

E Dialogue gaps

Pages 10-11

1. strength. Turn 2. Strength identifies product content per unit or quantity, and the current unclear field cannot be resolved from label color.

2. current order. Turn 4. The current order is the prescription being processed now, which needs its actual instructions clarified.

3. historical record. Turn 6. A historical record may provide context for the pharmacist but does not automatically replace unclear current instructions.

4. dose. Turn 8. Dose concerns the prescribed amount used for an administration, while strength describes the product's content per unit or quantity.

5. clarification request. Turn 10. The clarification request identifies the unresolved field and asks the appropriate professional to establish the intended current information.

6. pharmacist. Turn 12. The pharmacist is the identified professional route for resolving the order question rather than leaving the technician to select a strength.

7. ambiguous entry. Turn 14. The ambiguous entry remains unresolved, and its clarification process has no confirmed completion time in the supplied facts.

8. verified instruction. Turn 16. A verified instruction is the appropriately confirmed direction needed to resolve the order, not an expected answer or timing promise.

9. directions for use. Turn 18. Directions for use concern the authorized use of the medicine and should be addressed through the pharmacist's appropriate clinical discussion.

10. intake. Turn 20. Intake organizes the current request and its sources while keeping the unresolved field visible for the proper clarification process.

F Transfer conversation

Page 12

1. dosage form

Technician: "The unclear field concerns the dosage form."

The unresolved field is the product's physical form, not a verified strength.

2. white box

Patient: "I remember a white box."

The white box is reported memory and does not establish the product form.

3. Ren

Technician: "The clarification was accepted by Ren."

Ren is the named pharmacist who accepted the question.

4. unconfirmed

Technician: "Completion timing remains unconfirmed."

The brief supplies no confirmed completion time for the clarification or order.

02 Briefing and language checks

Insurance claims in plain English

A Briefing checks / page 13

1. What does the current response say?

D. Prior authorization required

The response identifies a prior-authorization requirement rather than a final permanent exclusion.

2. What is still absent?

A. An authorization decision and an accepted claim

The brief explicitly leaves the decision and claim acceptance unconfirmed.

3. Who accepted the status query?

D. Inez

Inez is the identified billing colleague handling the query and explaining its route.

D Language checks / page 18

1. Which explanation is accurate?

B. The plan requires prior authorization; no decision or accepted claim has arrived.

The explanation preserves the actual requirement and open outcome without inventing permanent denial or approval.

2. Which statement overstates the billing handoff?

B. Inez accepted the query, so the claim is now approved.

Accepting a status query is an administrative action and does not establish a plan decision.

3. Which statement accurately separates the request from the result?

A. Submission does not establish that authorization was granted.

Submitting a request begins or advances a process but does not establish the result.

4. What should happen to a question about a different medicine?

D. Refer it to the pharmacist for appropriate clinical discussion.

A payment response does not authorize the technician to make a clinical substitution.

02 Conversation answers

Insurance claims in plain English

E Dialogue gaps

Pages 19-20

- 1. prior authorization.** Turn 2. Prior authorization is the plan approval requirement stated in the response, not a completed decision or permanent exclusion.
- 2. coverage decision.** Turn 4. The coverage decision is the plan's actual determination, which has not arrived and cannot be assumed from submitting a form.
- 3. claim response.** Turn 6. The claim response is the recorded message requiring prior authorization; it should be explained without changing its meaning.
- 4. status query.** Turn 8. The status query has an identified owner, Inez, but accepting it does not resolve the coverage requirement.
- 5. accepted claim.** Turn 10. An accepted claim is a processing result that must be verified separately from a colleague's inquiry or contact attempt.
- 6. out-of-pocket cost.** Turn 12. Out-of-pocket cost is the patient's payment amount, which cannot be calculated or guaranteed from this requirement message alone.
- 7. copayment.** Turn 14. A copayment is a fixed payment under a particular benefit, not evidence of another patient's coverage or charge.
- 8. appeal.** Turn 16. An appeal reviews a relevant adverse decision under the applicable process; the current requirement does not establish that this is already the right step.
- 9. pharmacist.** Turn 18. The pharmacist is the appropriate route for a clinical alternatives question; insurance processing does not authorize the technician to choose treatment.
- 10. health plan.** Turn 20. The health plan's actual rules and decision govern the coverage process; the technician's explanation does not create approval or a deadline.

F Transfer conversation

Page 21

1. quantity limit

Technician: "The returned requirement concerns a quantity limit."

The response specifically identifies a quantity limit rather than a completed exception decision.

2. Sam

Patient: "The billing query is with Sam."

Sam is the colleague who accepted the status query.

3. pharmacist

Technician: "Any clinical change should be discussed with the pharmacist."

The supplied role assigns treatment questions to the pharmacist, not the billing response.

4. unconfirmed

Technician: "The exception outcome remains unconfirmed."

No exception decision or accepted claim has arrived, so the outcome is still open.

03 Briefing and language checks

Refill requests and response windows

03

A Briefing checks / page 22

1. What does the refill record show?

C. No refills remaining

The available record has no remaining refills and the renewal response is pending.

2. When was the renewal request sent?

C. 09:00

The brief gives 09:00 as the sent time and 11:00 as the current inquiry time.

3. What can Jo commit to?

C. A status call between 15:00 and 16:00

Jo controls the stated update call, not the prescriber's response or supply outcome.

D Language checks / page 27

1. Which update is accurate at 11:00?

A. The 09:00 renewal request is unanswered; Jo will give a status update between 15:00 and 16:00.

The accurate version preserves the request time, unresolved response, and separate communication commitment.

2. What does a successful transmission not prove?

D. Prescriber authorization

Transmission records communication activity rather than the clinical or legal authorization sought.

3. What should happen to a concern about running out?

B. Refer it promptly to the pharmacist for appropriate assessment.

The pharmacist must address the actual need and applicable options; the status-call window is not clinical advice.

4. Who owns the promised call?

A. Jo

Jo explicitly commits to the status call and should record its actual contact outcome.

03 Conversation answers

Refill requests and response windows

03

E Dialogue gaps

Pages 28-29

- 1. refills remaining.** Turn 2. Refills remaining describes existing authorization in the record; zero means the requested further supply is not already authorized there.
- 2. pending response.** Turn 4. A pending response means no reply has arrived; it does not establish a refusal or a change in treatment.
- 3. request timestamp.** Turn 6. The request timestamp records nine o'clock as the sending time and separates an actual sent request from a planned action.
- 4. Transmission confirmation.** Turn 8. Transmission confirmation can establish message movement but does not establish that the prescriber reviewed and approved the renewal.
- 5. status call.** Turn 10. The status call provides current information and does not guarantee that the order will be ready for collection.
- 6. callback window.** Turn 12. The callback window is Jo's agreed three-to-four communication period, independent of whether the renewal has been resolved.
- 7. treatment interruption.** Turn 14. Treatment interruption can raise a clinical concern that needs prompt pharmacist assessment rather than a routine update assumption.
- 8. emergency supply.** Turn 16. Emergency supply depends on the applicable clinical and legal requirements and is not automatically available from a technician's promise.
- 9. contact outcome.** Turn 18. Contact outcome distinguishes a successful conversation from an attempt or message, so the record does not falsely imply the patient was informed.
- 10. follow-up owner.** Turn 20. The follow-up owner is Jo for the agreed status call, not the person who can guarantee the prescriber's response.

F Transfer conversation

Page 30

1. 10:30

Technician: "The request was sent at 10:30."

The supplied sending time is 10:30, not the current inquiry time.

2. no reply

Patient: "The current status at 12:30 is no reply."

No prescriber reply has arrived at the stated current time.

3. 16:00 and 17:00

Technician: "Mei will call between 16:00 and 17:00."

That is the agreed communication window, not a collection guarantee.

4. unconfirmed

Patient: "The collection time remains unconfirmed."

The brief supplies no confirmed collection time despite the promised status call.

04 Briefing and language checks

Stock enquiries and delivery estimates

04

A Briefing checks / page 31

1. What is the current local stock position?

C. No stock for this order

The brief states that the pharmacy has no stock for the patient's order.

2. What does Wednesday describe?

A. The wholesaler's unconfirmed delivery estimate

Wednesday is a forecast from the supplier, with shipment still unconfirmed.

3. When can Lee check the supplier update?

A. Tuesday at 16:00

The stated next information checkpoint is Tuesday at 16:00.

D Language checks / page 36

1. Which statement preserves the facts?

A. We have no stock; Wednesday is estimated, shipment is unconfirmed, and Lee will check Tuesday at 16:00.

The statement separates the current shortage, supplier forecast, shipment gap, and information checkpoint.

2. What does carrier movement not establish by itself?

D. Collection readiness at the pharmacy

Transport progress does not establish receipt and completion of the remaining pharmacy steps.

3. Who should address an alternative medicine question?

D. The pharmacist under the applicable process

Alternative treatment or substitution requires the appropriate professional and legal process.

4. What does Tuesday at 16:00 represent?

C. A supplier information check, not guaranteed supply or clinical advice to wait

The brief gives a checkpoint for information and does not establish a supply outcome or safe waiting period.

04 Conversation answers

Stock enquiries and delivery estimates

E Dialogue gaps

Pages 37-38

1. stockout. Turn 2. The stockout describes the pharmacy's current lack of the required product, not a confirmed shortage at every pharmacy.

2. supplier estimate. Turn 4. The supplier estimate forecasts Wednesday delivery without guaranteeing a morning arrival or a patient collection time.

3. shipment confirmation. Turn 6. Shipment confirmation has not arrived; that gap is not itself a definite cancellation or completed dispatch.

4. supplier checkpoint. Turn 8. The supplier checkpoint is Tuesday at sixteen hundred, when Lee can check updated information rather than guarantee delivery.

5. Carrier tracking. Turn 10. Carrier tracking records shipment movement and does not establish that the product has reached the pharmacy or completed its checks.

6. order allocation. Turn 12. Order allocation identifies the quantity assigned to this particular order rather than a general delivery or stock announcement.

7. partial delivery. Turn 14. A partial delivery contains less than the ordered quantity and should not be reported as complete fulfillment.

8. therapeutic alternative. Turn 16. A therapeutic alternative is a clinical option requiring appropriate assessment rather than a technician's stock-based replacement choice.

9. regional shortage. Turn 18. A regional shortage concerns availability across an area and cannot be inferred solely from this pharmacy's local stockout.

10. Collection readiness. Turn 20. Collection readiness requires the relevant receipt and pharmacy checks, not just a supplier forecast or movement in transit.

F Transfer conversation

Page 39

1. Friday

Technician: "Estimated delivery is Friday."

Friday is the supplier's forecast, not a guaranteed collection time.

2. unconfirmed

Patient: "Dispatch is still unconfirmed."

The brief explicitly leaves dispatch unconfirmed despite the estimate.

3. Thursday at 11:00

Technician: "The supplier check is Thursday at 11:00."

Thursday 11:00 is the stated information checkpoint, not a collection promise.

4. pharmacist

Patient: "An alternative-treatment question goes to the pharmacist."

The pharmacist is the appropriate route for the clinical alternatives discussion.

05 Briefing and language checks

Pickup questions and pharmacist consultations

A Briefing checks / page 40

1. What does Arden ask about?

D. An auxiliary label

The question concerns the meaning of an additional label on the medicine package.

2. Who has accepted the question?

C. Pharmacist Ravi

Ravi is available at the consultation counter and has accepted the handoff.

3. What changes to the instructions are established?

B. None

The scenario explicitly states that no instructions are changed.

D Language checks / page 45

1. Which response completes the appropriate handoff?

A. Ravi has accepted your auxiliary-label question and is available at the consultation counter.

The response identifies the question, confirmed pharmacist owner, and practical location without changing the instruction.

2. What does Ravi's acceptance establish?

D. The question has a receiving pharmacist, not a completed consultation.

Acceptance confirms the handoff while the actual discussion and understanding remain separate.

3. Which statement is inappropriate?

C. The auxiliary label is always optional.

An additional label can contain important directions or cautions and cannot be dismissed as optional by category.

4. What should the patient be encouraged to clarify?

D. The actual unclear wording and their understanding during the pharmacist discussion

The pharmacist needs the real instruction and question to provide an appropriate explanation.

05 Conversation answers

Pickup questions and pharmacist consultations

E Dialogue gaps

Pages 46-47

1. auxiliary label. Turn 2. The auxiliary label is the additional label prompting the question; identifying it is different from interpreting its instruction.

2. dispensing label. Turn 4. The dispensing label and additional information may have different functions; the main label does not make every auxiliary instruction optional.

3. patient counseling. Turn 6. Patient counseling is the professional discussion needed to understand the instruction, not a hurried interpretation by an unauthorized technician.

4. consultation counter. Turn 8. The consultation counter gives Arden a specific location for the pharmacist who has accepted the question.

5. accepted handoff. Turn 10. An accepted handoff confirms that Ravi will receive the question, not that the counseling discussion has already occurred.

6. patient information leaflet. Turn 12. The patient information leaflet is another information source the pharmacist can discuss in relation to the actual label and question.

7. administration device. Turn 14. An administration device relates to delivering the medicine and should be discussed within the appropriate pharmacist consultation and role.

8. privacy. Turn 16. Privacy means handling the personal question through appropriate discussion arrangements rather than exposing it to the queue.

9. teach-back. Turn 18. Teach-back checks the person's understanding so the professional can clarify the explanation; it is not proof of understanding from a simple yes.

10. counseling completion. Turn 20. Counseling completion concerns the actual professional discussion, which must not be recorded merely because the referral or package handover occurred.

F Transfer conversation

Page 48

1. storage

Technician: "The question concerns the storage wording."

The supplied question concerns storage, not an invented change to dosing.

2. Noor

Patient: "The pharmacist who accepted it is Noor."

Noor is the identified pharmacist who accepted the question.

3. consultation

Technician: "Please use the consultation area."

The pharmacist is available in the specified consultation area.

4. unchanged

Technician: "The directions remain unchanged."

The scenario explicitly states that no instruction has been changed.

05

06 Briefing and language checks

Private collection and contact preferences

A Briefing checks / page 49

1. What is unresolved?

D. The collection arrangement under the pharmacy process

The case establishes an unverified arrangement, not wrongdoing or a universal prohibition.

2. Who can review the request privately?

A. Lead pharmacist Maya

Maya is the available pharmacist identified to review this request.

3. Which requested action should Vera avoid?

C. Reading the medication list aloud as an improvised identification test

Naming medicines publicly would reveal information without resolving the collection arrangement through the appropriate process.

D Language checks / page 54

1. Pavel says, "So friends are never allowed to collect?" Which response is accurate?

B. That is not what I am saying; Maya can review this unresolved request privately.

The response distinguishes this unresolved request from a universal rule and gives a confirmed review route.

2. Why does confirming Pavel's identity not finish this review?

B. It establishes who he is, while this collection arrangement still needs review.

Identity alone does not determine every collection or disclosure decision.

3. What can Vera promise now?

C. Connection with Maya for private review

Maya is available to review; the decision and contact permissions remain unestablished.

4. Which note is factual?

A. Neighbor collection requested; arrangement unresolved; referred to Maya privately.

The note records established events without inventing wrongdoing, legal requirements, or patient permission.

06 Conversation answers

Private collection and contact preferences

E Dialogue gaps

Pages 55-56

1. collection arrangement. Turn 2. The collection arrangement remains unresolved, so Vera names the process needing review rather than inventing approval or misconduct.

2. medication list. Turn 4. The medication list contains patient information and should not be read publicly as an improvised identity test.

3. pickup request. Turn 6. A pickup request records the action Pavel wants; its existence does not establish that the collection decision is complete.

4. professional judgment. Turn 8. Professional judgment applies relevant circumstances and rules; uncertainty does not justify inventing a universal ban on friend pickups.

5. identity verification. Turn 10. Identity verification concerns who Pavel is; it does not independently resolve every question about receiving another person's prescription.

6. contact permission. Turn 12. Contact permission must be established through the actual process rather than assumed from a number offered by another person.

7. private review. Turn 14. A private review moves the unresolved request to an appropriate setting without promising that collection will be approved.

8. relevant information. Turn 16. Relevant information is limited to the actual request and unresolved arrangement, not an invented demand for unrestricted patient records.

9. confidentiality. Turn 18. Confidentiality requires appropriate protection of information while the request follows its proper review route, not public accusation or disclosure.

10. authority to collect. Turn 20. Authority to collect is the unresolved basis for receiving the prescription; it must not be assumed from the request alone.

F Transfer conversation

Page 57

1. telephone number

Technician: "The new telephone number has not been verified."

The supplied number needs verification through the actual contact process.

2. unverified

Collector: "Contact permission is still unverified."

The facts do not establish permission to use this contact.

3. Ellis

Technician: "Pharmacist Ellis has accepted the review."

Ellis is the pharmacist explicitly identified as the receiving owner.

4. disclosed

Technician: "No patient information has been disclosed."

The scenario states that patient information has not been disclosed.

07 Briefing and language checks

Precise inventory and quality queries

A Briefing checks / page 58

1. Which source has not been checked?

C. The label on the actual stock

The case explicitly states that the stock label has not been checked.

2. What can be concluded now?

A. A discrepancy needs assessment; affected-stock status is not established.

The two records differ, but actual stock and full notice scope still need review.

3. Who owns the recall assessment?

C. Pharmacist Maya

Maya is the named professional responsible for this assessment.

D Language checks / page 63

1. Which read-back preserves the notice code?

D. A, B, four, one

AB41 belongs to the notice; AB14 belongs to the delivery record.

2. Which statement overstates the evidence?

A. The shelf is clear because the delivery record shows a different lot.

A delivery entry alone does not prove which lot is physically present or whether stock falls within scope.

3. What should be preserved?

C. The source identifiers, actual findings, and authorized decisions

Traceability depends on accurate source references and a record of what was actually checked and decided.

4. Which patient-facing claim is unsupported?

C. Every recall means you should stop taking medicine immediately.

Treatment actions depend on the specific recall and professional advice, not a universal instruction invented from the word recall.

07 Conversation answers

Precise inventory and quality queries

E Dialogue gaps

Pages 64-65

- 1. recall notice.** Turn 2. The recall notice is the source naming AB41; its identifier must remain distinct from the different delivery entry.
- 2. lot number.** Turn 4. A lot number identifies the production group and must be read in the correct character order to support matching.
- 3. delivery record.** Turn 6. The delivery record documents an entry but does not alone establish the identifier on the stock physically present.
- 4. physical stock.** Turn 8. Physical stock is the inventory actually present; checking it is distinct from reading a delivery entry.
- 5. recall scope.** Turn 10. Recall scope defines the relevant products and limits; a single similar code does not replace the complete notice comparison.
- 6. Patient exposure.** Turn 12. Patient exposure concerns whether someone received or used relevant product and is not established merely by an inventory query.
- 7. traceability.** Turn 14. Traceability depends on retaining source references and verified findings so product history and decisions can be followed.
- 8. Quarantine.** Turn 16. Quarantine controls availability pending an authorized decision; it is not automatically a return, destruction, or final release decision.
- 9. disposition.** Turn 18. Disposition is the decision about the product's handling; it does not authorize a technician to invent patient treatment advice.
- 10. release decision.** Turn 20. A release decision requires the authorized assessment and cannot be inferred from the differing delivery code alone.

F Transfer conversation

Page 66

1. CD27

Technician: "The notice names CD27."

CD27 is the notice identifier, not the different delivery entry.

2. CD72

Pharmacist: "The delivery entry is CD72."

CD72 is the delivery identifier and must not be transposed.

3. unchecked

Technician: "The actual stock is still unchecked."

No check of actual stock has yet been established.

4. Noor

Pharmacist: "The assessment owner is Noor."

Noor is the explicitly named owner of this assessment.

08 Briefing and language checks

Corrected messages and shift handoffs

A Briefing checks / page 67

1. What was wrong with the 14:00 message?

A. It said ready although pharmacist review was still pending.

The ready statement was based on a misread queue entry and must be withdrawn.

2. Which time is an update commitment?

B. 16:00

Jo owns a 16:00 status update, not a guaranteed collection or review completion.

3. What must a handoff distinguish?

D. Attempted contact and a correction actually communicated

An attempted call does not establish that the patient received the corrected information.

D Language checks / page 72

1. Which opening most clearly repairs the error?

D. I need to correct my 14:00 ready message: pharmacist review is still pending.

The response explicitly replaces the inaccurate statement with the verified pending status.

2. Which promise is supported?

A. Jo will provide a status update at 16:00.

The supplied commitment concerns information at 16:00, not dispensing completion or receipt.

3. If the call is unanswered, what should the record show?

D. An attempted contact with the correction action still unresolved

An unanswered call does not establish delivery or acknowledgment of the correction.

4. Which handoff is complete in principle?

D. Verified status, actual contact outcome, open follow-up, and accepting owner

A closed-loop handoff preserves facts, outstanding work, and confirmed responsibility.

08 Conversation answers

Corrected messages and shift handoffs

E Dialogue gaps

Pages 73-74

1. ready notification. Turn 2. The ready notification prompted travel, so the correction must explicitly replace that earlier claim rather than offer a vague apology.

2. verified status. Turn 4. The verified status is pending pharmacist review, not ready for collection; it replaces the earlier inaccurate message.

3. pharmacist review. Turn 6. Pharmacist review is the outstanding professional step and must not be represented as complete merely because a queue entry exists.

4. update commitment. Turn 8. The update commitment concerns a 16:00 status report; it is not a replacement guarantee that collection will be possible then.

5. readiness estimate. Turn 10. A readiness estimate forecasts availability, whereas the only supplied 16:00 commitment is to report status.

6. successful contact. Turn 12. Successful contact means the communication actually reached the intended person, unlike an unanswered call or prepared message.

7. open action. Turn 14. The open action is the unfinished follow-up and must remain visible with an owner rather than disappear at a shift change.

8. closed-loop handoff. Turn 16. A closed-loop handoff confirms receipt and ownership, not merely a note naming someone who may never have accepted the task.

9. audit trail. Turn 18. The audit trail preserves the original event and subsequent correction so later reviewers can understand the sequence accurately.

10. status correction. Turn 20. The status correction replaces the inaccurate ready statement while preserving the pending review and the separate follow-up commitment.

F Transfer conversation

Page 75

1. sent

Erin: "The inaccurate message has not been sent."

The message is prepared but has not reached the patient.

2. pending

Colleague: "The verified review status is pending."

Review remains pending, so readiness must not be asserted.

3. Erin

Erin: "The correction owner is Erin."

Erin explicitly owns correcting the queued message before release.

4. release

Colleague: "Correct it before message release."

The message must be corrected before it is released to the patient.

08

09 Conversation answers

Check what the number measures

E Extended conversation

Pages 76-77

1. directions for use. Turn 2. The verified current directions specify one tablet twice daily; the previous once-daily pattern cannot determine this order's days supply.

2. days supply. Turn 4. Sixty tablets divided by two tablets per day equals thirty days; the copied sixty-day field does not match the current directions.

3. strength. Turn 6. Strength is five milligrams per tablet; it is not the number of tablets used each day and cannot replace the daily tablet quantity in this calculation.

4. clinical approval. Turn 8. Clinical approval and the required pharmacy checks are separate from arithmetic; a correct quotient does not establish that a prescription is appropriate or ready to supply.

5. maximum daily use. Turn 10. Thirty tablets divided by the explicit maximum of six daily equals five days at maximum use, not a prediction that the patient will consume that amount every day.

6. Adherence. Turn 16. Adherence concerns following the agreed medicine directions; a quantity-based calculation or refill interval alone does not prove how the patient actually used the medicine.

F Transfer exchange

Page 78

1. 3

Technician: The scheduled order uses 3 tablets daily.

One tablet on each of three daily occasions equals three tablets per day, not ten.

2. 28

Pharmacist: Eighty-four tablets at that rate represents 28 days.

Eighty-four divided by three tablets per day equals twenty-eight days under the supplied schedule.

3. 6

Technician: The separate as-needed quantity represents 6 days at maximum use.

Twenty-four tablets divided by the maximum of four daily equals six days at that maximum.

4. unknown

Pharmacist: Actual as-needed consumption remains unknown .

The maximum-use calculation supplies a conditional rate, not evidence of the amount actually taken.

Background reading: [PTCB: 2026 knowledge reference, calculations and pharmacist intervention](#). The conversation is an original fictional teaching case.

10 Conversation answers

Read the whole temperature log

E Extended conversation

Pages 79-80

1. temperature excursion. Turn 2. The excursion is the recorded interval outside this fictional product's two-to-eight-degree range; the current in-range reading does not erase that history.

2. thirty-five minutes. Turn 4. The established interval from 10:05 to 10:40 lasts thirty-five minutes; it is not the fifty-five minutes from the interval's start to the current 11:00 reading.

3. air temperature. Turn 6. The sensor records fridge air, not a directly measured vial-content temperature; the assessment must preserve that measurement distinction.

4. traceability. Turn 8. Traceability connects the affected quantity, product and lot with the specific storage event and subsequent authorized decision.

5. Quarantine. Turn 10. Quarantine prevents use pending assessment; it is not permission to abandon required storage conditions or a final decision to destroy the medicine.

6. disposition. Turn 16. Disposition is the authorized decision about use, return, or disposal after assessment; neither an excursion nor a normal current reading establishes that decision by itself.

F Transfer exchange

Page 81

1. 30

Technician: The established excursion lasted 30 minutes.

The interval from 14:20 to 14:50 is thirty minutes, not forty minutes to the current reading.

2. 10.2

Pharmacist: The recorded peak was 10.2 degrees Celsius.

The historical peak is 10.2; the current 4.8 reading must not replace it in the event report.

3. vials

Technician: Two cartons of fifteen means thirty vials on hold.

The quantity unit is vials: two cartons multiplied by fifteen vials per carton equals thirty vials.

4. pending

Pharmacist: The authorized disposition is still pending .

The supplied facts establish a hold and current reading, not a completed decision about release or disposal.

10

Background reading: [NHS SPS: managing temperature excursions and product-specific assessment](#). The conversation is an original fictional teaching case.

11 Conversation answers

A change before the next pack

E Extended conversation

Pages 82-83

1. multi-compartment pack. Turn 2. The multi-compartment pack for 12 to 18 October still contains the discontinued medicine and must enter the agreed hold and correction process before release.

2. effective date. Turn 4. The effective date is Friday 9 October, not the Monday start of the next packaging cycle; the change affects the current pack as well as future preparation.

3. pack identifier. Turn 6. The pack identifier and covered dates distinguish the held future pack from the current pack at the patient's home.

4. Medicines reconciliation. Turn 10. Medicines reconciliation compares the actual medicine information and resolves discrepancies; receiving a changed order does not prove that all packs, records, and patient instructions already match.

5. prepared. Turn 14. Prepared describes work on a pack; it is not equivalent to final checking, release, dispatch, delivery, or the patient receiving instructions.

6. contact outcome. Turn 18. Contact outcome distinguishes an attempted call from successful communication and an agreed arrangement; accepting responsibility does not itself prove that the patient has been reached.

F Transfer exchange

Page 84

1. 14

Technician: The change takes effect on 14 October.

The verified effective date is the fourteenth, not the next pack's nineteenth-of-October start date.

2. 18

Pharmacist: The current pack ends on 18 October.

The current pack covers the twelfth through the eighteenth and therefore spans the change date.

3. 19

Technician: The next pack begins on 19 October.

The undelivered next pack starts on the nineteenth; that does not postpone the earlier effective change.

4. completed

Pharmacist: Patient communication is not yet confirmed as completed .

A named pharmacist and a verified order do not establish that the patient has received the updated instructions.

11

Background reading: [NHS SPS: complex regimens and avoiding confusion between old and new medicine packs](#). The conversation is an original fictional teaching case.

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Prescription intake without assumptions

Name the unresolved field: The strength field on this order is unclear.

State the role: I cannot select a strength from an unclear entry.

Distinguish terms: Strength describes the product; dose concerns the prescribed amount used.

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Insurance claims in plain English

Read the status: The response says prior authorization required.

Avoid false reassurance: It also does not mean approval is guaranteed.

Keep the price unconfirmed: I cannot confirm your final payment from this response alone.

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Refill requests and response windows

State the record: The record shows no refills remaining.

Keep stages separate: Sending the request does not mean the renewal is approved.

Keep supply unconfirmed: No further supply has been confirmed yet.

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Stock enquiries and delivery estimates

State current availability: We do not currently have stock for your order.

Avoid a collection promise: I cannot confirm a pickup time from that estimate alone.

Check quantity: We need the quantity for this order confirmed as well as timing.

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 Pickup questions and pharmacist consultations

Welcome the question: Thank you for asking about that label.

Name the pharmacist: Ravi has accepted your question.

Do not change instructions: I am not changing any direction on this package.

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 Private collection and contact preferences

Acknowledge the purpose: I understand that you have come to help your neighbor.

Protect the list: I cannot use a public reading of the medication list as an identification test.

Keep the patient central: We need to handle the patient's information through the appropriate process.

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Precise inventory and quality queries

Name the document: The notice names lot AB41.

State the missing check: The actual stock label has not yet been checked.

Separate records and stock: A delivery entry is not a physical-stock check.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Corrected messages and shift handoffs

Open the correction: I need to correct the ready message I gave you at 14:00.

Apologize plainly: I am sorry that my message caused this disruption.

Keep the distinction: That is an update time, not a pickup appointment.

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

accepted claim	verify an accepted claim	14
accepted handoff	confirm an accepted handoff	42
acknowledgment	obtain acknowledgment	69
active ingredient	identify the active ingredient	5
adherence	discuss an adherence concern	24
adjudication	check claim adjudication	14
administration device	identify the administration device	42
affected stock	identify affected stock	60
alcohol interaction	refer an alcohol-interaction question	42
allocation	verify the allocation	32
alternate contact	verify an alternate contact	50
ambiguous entry	flag an ambiguous entry	6
appeal	clarify the appeal route	15
audit trail	maintain the audit trail	69
authority to collect	clarify authority to collect	50
authorization pending	explain authorization pending	23
authorization record	check an authorization record	51
auxiliary label	identify the auxiliary label	41
available stock	confirm available stock	32
backorder	track a backorder	32
batch	trace the batch	59
benefit tier	identify the benefit tier	14
brand name	confirm the brand name	6
callback number	verify the callback number	51

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

callback window confirm the callback window	23
carrier tracking check carrier tracking	32
claim submit a claim	14
claim response explain the claim response	14
clarification request route a clarification request	6
closed-loop handoff use a closed-loop handoff	69
coinsurance explain coinsurance	15
collected record the item as collected	68
collection arrangement verify the collection arrangement	50
collection readiness verify collection readiness	33
collection time confirm collection time	23
concentration verify the concentration	5
confidentiality maintain confidentiality	50
consultation counter direct to the consultation counter	41
contact attempt record a contact attempt	68
contact outcome record the contact outcome	24
contact permission verify contact permission	50
continuity of therapy support continuity of therapy	24
contraindication refer a contraindication question	41
copayment verify the copayment	15
correction owner name the correction owner	69
counseling completion record counseling completion	42
coverage decision confirm the coverage decision	14
coverage exclusion verify a coverage exclusion	14

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

current order verify the current order	6
days supply verify days supply	6
deductible verify the deductible	15
delivery record reconcile the delivery record	59
delivery window clarify the delivery window	32
designated collector confirm the designated collector	50
destruction record retain a destruction record	60
directions for use verify directions for use	5
disclosure limit the disclosure	50
dispatch confirm dispatch	32
dispensed distinguish dispensed from collected	68
dispensing label verify the dispensing label	41
disposition record the disposition	60
dosage form verify the dosage form	5
dose distinguish dose from strength	5
drowsiness report drowsiness	42
emergency supply refer an emergency-supply question	24
exception request route an exception request	15
expiration date check the expiration date	59
final check confirm the final check	68
first-expiry-first-out apply first-expiry-first-out	60
follow-up owner name the follow-up owner	24
food interaction refer a food-interaction question	41
formulary check the formulary	14

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

frequency clarify the frequency	5
generic name identify the generic name	6
generic substitution refer a generic-substitution question	33
handoff record complete the handoff record	69
health plan verify the health plan	14
historical record consult the historical record	6
identity mismatch resolve an identity mismatch	51
identity verification complete identity verification	50
in progress mark the task in progress	68
instruction interpretation refer instruction interpretation	42
intake complete prescription intake	5
interaction ask about an interaction	41
interchangeability verify interchangeability	33
lead time qualify the lead time	33
light protection clarify light-protection wording	42
lot number read back the lot number	59
manufacturer confirm the manufacturer	59
measuring device identify the supplied measuring device	42
medication guide provide the medication guide	41
medication list protect the medication list	51
member identifier verify the member identifier	15
message delivered confirm the message was delivered	69
National Drug Code compare the National Drug Code	59
NDC check the NDC	6

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

open action	assign the open action	69
order allocation	check order allocation	33
order field	clarify an order field	6
out-of-pocket cost	confirm out-of-pocket cost	15
package insert	identify the package insert	41
partial delivery	identify a partial delivery	33
partial fill	refer a partial-fill question	24
patient counseling	arrange patient counseling	41
patient exposure	assess patient exposure	60
patient information leaflet	identify the patient information leaflet	41
patient preference	record a patient preference	50
PBM	identify the relevant PBM	15
pending	show the review as pending	68
pending response	track a pending response	24
pharmacist	refer to the pharmacist	5
pharmacist review	await pharmacist review	68
pharmacy network	verify pharmacy network status	15
physical stock	inspect the physical stock	59
pickup request	review a pickup request	51
precaution	ask about a precaution	41
preferred channel	confirm the preferred channel	50
prescriber	contact the prescriber	5
prescriber response	await the prescriber response	23
prescription	verify a prescription	5

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

prescription transfer clarify prescription-transfer requirements	24
prior authorization request prior authorization	14
privacy protect privacy	42
private review arrange a private review	51
product availability confirm product availability	33
product identifier verify the product identifier	6
product identifier match the product identifier	59
professional judgment exercise professional judgment	50
public counter avoid details at the public counter	51
quality defect report a quality defect	60
quantity confirm the quantity	6
quantity limit verify a quantity limit	15
quarantine apply the quarantine process	60
queue entry check the queue entry	68
readiness estimate qualify a readiness estimate	69
ready notification withdraw a ready notification	68
reasonable inference make a reasonable inference	50
recall notice review the recall notice	59
recall scope establish the recall scope	60
receipt confirm receipt	32
refill request a refill	23
refill too soon explain a refill-too-soon response	24
refills remaining check refills remaining	23
refrigeration clarify a refrigeration instruction	42

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

regional shortage verify a regional shortage	33
rejected claim investigate a rejected claim	14
relationship claim assess the relationship claim	51
release decision document a release decision	60
relevant information share relevant information	51
remaining quantity verify the remaining quantity	24
renewal request a prescription renewal	23
renewal request send a renewal request	23
request timestamp verify the request timestamp	23
response code clarify the response code	14
response window qualify the response window	23
return authorization obtain return authorization	60
route verify the route	5
serial number verify a serial number	59
service recovery support service recovery	69
shipment confirmation obtain shipment confirmation	32
status call make a status call	23
status correction issue a status correction	68
status query accept a status query	15
step therapy clarify a step-therapy requirement	15
stock on hand verify stock on hand	32
stock reconciliation complete stock reconciliation	59
stockout report a stockout	32
storage condition clarify storage conditions	42

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

strength clarify the strength	5
successful contact document successful contact	69
supplier checkpoint confirm the supplier checkpoint	33
supplier estimate qualify the supplier estimate	32
supply authorization verify supply authorization	24
supply disruption report a supply disruption	33
tamper evidence check tamper evidence	60
teach-back use teach-back appropriately	42
temperature excursion report a temperature excursion	60
therapeutic alternative discuss a therapeutic alternative	33
third party verify a third-party request	51
timestamp preserve the timestamp	68
traceability preserve traceability	59
transmission confirmation check transmission confirmation	23
treatment interruption report a treatment interruption	24
unconfirmed forecast label an unconfirmed forecast	33
update commitment keep the update commitment	69
verified destination use a verified destination	51
verified instruction preserve the verified instruction	6
verified status communicate the verified status	69
voicemail check voicemail preferences	51
warning clarify a warning	41
wholesaler contact the wholesaler	32
workflow stage verify the workflow stage	68

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] Pharmacy Technician Certification Board. Guidebook: Code of Conduct.

[Open the original source](#)

United States credentialing context for truthful communication, confidentiality, supervision, and practice limits. Fictional role assignments do not establish universal technician duties. Checked 10 October 2026.

[2] HealthCare.gov. Prior Authorization.

[Open the original source](#)

United States insurance terminology. The original claim scenario does not establish any specific plan decision, response deadline, coverage, or patient charge. Checked 10 October 2026.

[3] U.S. Department of Health and Human Services. Prescription Pickup by a Friend or Family Member.

[Open the original source](#)

United States privacy guidance permits appropriate pickup using pharmacist judgment. The fictional unresolved request is not a blanket ban or universal written-authorization requirement. Checked 10 October 2026.

[4] U.S. Food and Drug Administration. Understanding Drug Recalls.

[Open the original source](#)

Background on matching recall details and obtaining appropriate guidance. The fictional lot comparison supplies no affected-stock finding or patient medication instructions. Checked 10 October 2026.

Phil Stilwell

© 2026 Phil Stilwell. All rights reserved.

No part of this publication may be reproduced, stored in a retrieval system, or transmitted in any form or by any means without prior written permission from the author, except as permitted by law.

All cases, calculations, and product limits are fictional language exercises, not patient-specific directions or clinical certification. Follow local law, authorized duties, pharmacist direction, privacy procedures, and actual recall notices. Urgent concerns need the appropriate prompt response, not a routine update window. US and UK sources provide context, not interchangeable rules. This book does not teach prescribing, dose selection, or clinical counseling.

English Ladder | English for Work | 2026 edition. The B2-C1 range is an editorial study recommendation, not a proficiency certification.

[Open the matching English Ladder course](#)